

A healthcare worker wearing a blue protective suit, a face shield, and a white mask is shown in a clinical setting. They are holding a white test tube in their right hand and gesturing with their left hand, which is wearing a blue nitrile glove. The background is a plain, light-colored wall.

WHO procurement report 2025



World Health
Organization

WHO procurement report 2025

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Cover Photo: Strengthening lab capacity: Influenza–SARS-CoV-2 real-time RT-PCR test in Cambodia - © WHO / Enric Catala Contreras.

In Cambodia, WHO works with the Ministry of Health's National Reference Medical Laboratory (NRML) to build technical capacity on Influenza–SARS-CoV-2 real-time RT-PCR testing.

A WHO-supported training session on molecular testing to detect Influenza and SARS-Cov2 for laboratory personnel from four provincial hospitals (Battambang, Kampong Cham, Siem Reap, and Preah Sihanouk) built technical capacity to collect quality respiratory specimens, perform real-time RT-PCR tests, and properly manage waste.

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Introduction

Founded in 1948, the World Health Organization (WHO) is the United Nations agency that connects nations, partners and people to promote health, keep the world safe and serve the most vulnerable – so everyone, everywhere can attain the highest level of health.

WHO leads global efforts to expand Universal Health Coverage. It directs and coordinates the world's response to health emergencies. And it promotes healthier lives – from pregnancy care through to old age. WHO's Triple Billion targets outline an ambitious plan for the world to achieve good health for all, using evidence-based policies and programmes. WHO procures and supplies on a yearly basis a significant amount of goods and services to enable its important public health mandate, and procurement is therefore a critical function of the organization.

The WHO procurement report, which has been published since 2020, provides an overview of procurement at WHO, details essential procurement data for the year, and showcases key initiatives and highlights. This report is produced in line with WHO's commitment to transparency and for the benefit of WHO Member States, donors, partners, WHO staff, the supplier community and other stakeholders.

The WHO procurement report complements the Annual Statistical Report on United Nations Procurement (ASR) to which WHO contributes and that provides an overview of procurement data of approximately 40 organizations within the United Nations (UN) system. The ASR is available on the United Nations Global Marketplace (UNGM) and includes interactive data dashboards *(1)*.

PART 1

Procurement at WHO



Myanmar earthquake response 2025 - © WHO.

Two powerful earthquakes devastated central Myanmar, disrupting health services & putting thousands at risk of life-threatening injuries and disease outbreaks. WHO responded at its highest level of emergency activation by deploying tonnes of emergency medical supplies to provide life-saving trauma care, prevent disease outbreaks & restore essential health services in Myanmar.

In order to fulfil its mandate and achieve its vision, WHO procures a significant volume of goods and services. In the period 2013–2019, WHO purchased on average US\$ 770 million worth of goods and services per year. In 2020, the procurement volume rose sharply to US\$ 1718 million due to the global pandemic of coronavirus disease 2019 (COVID-19) and the related emergency response provided by WHO. Since 2020, there has been a steady decline in WHO's total procurement towards pre-pandemic levels.

In 2025, the trend continued, as WHO procurement activities were undertaken in a context of heightened financial constraints and funding uncertainty, requiring strengthened stewardship over organizational resources and a cautious approach to expenditure.

WHO purchases in goods and services declined by 25% from US\$ 1391.2 million in 2024 to US\$ 1048.4 million in 2025. The decline was driven primarily by a reduction in the procurement of services, which decreased by 29% to US\$ 765.5 million, while the value of goods procured decreased by 12% to US\$ 282.9 million. Despite this decline, services remained the largest component of WHO procurement in 2025, accounting for 73% of total procurement value.

As a public organization entrusted with donor funds and committed to supporting developing economies, the objective of procurement and supply activities within WHO is the timely acquisition and delivery of goods and services, while respecting guiding principles for procurement.

The overall guiding principle for all WHO procurement is to obtain the best value for money for the Organization. "Best value for money" is defined as the most responsive offer that represents the best combination of technical specifications, quality and price. Best value for money is the result of several factors, including: quality; experience; suppliers' reputation; and life-cycle costs, benefits and parameters that measure how well the good or service allows the Organization to meet its objectives, and has the greatest possible impact on the outcomes from WHO programmes. Other guiding principles (2) that are the basis for WHO procurement are fairness, integrity, transparency and equal treatment; effective competition; sustainability; and the interest of WHO. As a public organization, WHO must also strictly adhere to the Organization's Financial Regulations and Rules and dedicated procurement policies and procedures, which mandate, among other requirements, that contracts be awarded through a competitive process, except when otherwise authorized by designated officials. Suppliers play an essential role in WHO's supply chain and in 2025 alone, WHO contracted over 12 935 different suppliers from 182 countries (for guidance on how to become a supplier to WHO refer to Annex 3).

At WHO, responsibility for procurement is based on a tiered system, with procurement carried out at the global, regional and local/country levels. In addition, the WHO Health Emergencies Programme has delegated authority for procurement for emergencies in accordance with established policies and protocols. Contract Review Committees at the WHO headquarters and regional offices ensure that procurement undertaken by WHO complies with the procurement principles and relevant policies and procedures, procurement risks are properly assessed and mitigated, and the best value for money and the interest of the organization are fully achieved. Furthermore, WHO procurement activities are continuously subject to rigorous internal and external audits.

PART 2

Procurement statistics



Outbreak of Ebola virus disease, Kasai Province, Democratic Republic of the Congo, September 2025 - © WHO / Joel Lumbala.

The Democratic Republic of the Congo declared an outbreak of Ebola virus disease on 4 September 2025, in the country's Kasai Province where a cluster of cases and deaths had been reported. The outbreak is the 16th in the country since the disease was first detected in 1976. In support of the government's response, WHO and partner organizations have mobilized experts to work with their national response teams to rapidly scale up efforts and end the outbreak as quickly as possible.

Key outbreak control measures including disease surveillance, clinical care, logistics, contact tracing and community engagement are being strengthened to curb the spread of the virus and protect communities.

Healthcare staff vaccination in Bulape.

This section covers key procurement statistics for the year that ended on 31 December 2025. Every procurement transaction is made and confirmed by issuing a Purchase Order (PO), which commits the relevant funds. The scope of the data in this report covers all WHO offices (except the Regional Office for the Americas¹) and contains all POs approved in 2025. The data are based on the same data set used to produce the statistics for the Annual Statistical Report on United Nations Procurement (ASR) to which WHO contributes annually. The ASR containing the data for the prior year is usually published in June or July and made available on the ASR page (1) on the United Nations Global Marketplace including through interactive dashboards. Hence, the data used to produce this WHO report, will also be available on the United Nations Global Marketplace from mid-2026 onwards.

In the ASR, procurement statistics have been collected from organizations in the United Nations System and have been reported together since 1984, with 2024 marking the 41st year of the United Nations system reporting on its procurement activities through the Annual Statistics Report. In 2024 (the most recent data currently available), the United Nations reported a collective US\$ 25.7 billion in procurement of goods and services. This represented an increase of US\$ 733 million, or 2.9%, compared with 2023, making 2024 the third-highest annual procurement volume on record. Health remained the top sector of UN procurement in 2024 at US\$ 5.3 billion, driven by increased procurement for pharmaceuticals, contraceptives, vaccines and health-care services.

The present report provides additional details and statistics on WHO procurement to complement data provided through the ASR.

2.1 Procurement data overview

To ensure sound financial management, transparency, and audit defensibility, the Organization introduced a series of early cost-containment measures at the start of 2025, designed to align procurement commitments with available financing while safeguarding essential programme delivery and emergency response.

The total value of procurement in 2025 was US\$ 1048.4 million (Fig. 1). Services remained the largest type of procurement, accounting for US\$ 765.5 million (73% of total procurement value), with goods accounting for US\$ 282.9 million (27% of total procurement).

WHO procurement has continued to decline from the peak reached in 2020, when procurement increased sharply to meet the unprecedented needs of the COVID-19 pandemic. This trend accelerated further in 2025, with a 25% decrease in overall procurement from US\$ 1391.2 million in 2024.

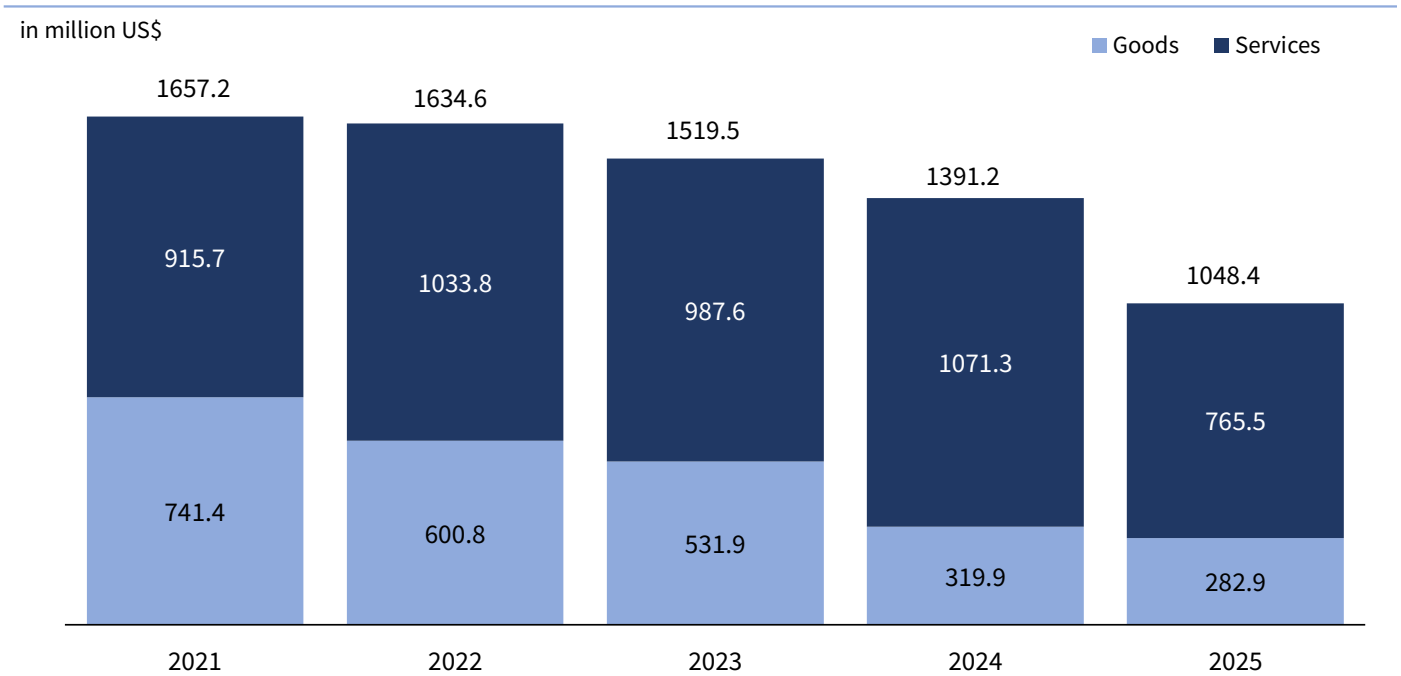
The decline in overall procurement value in 2025 was driven primarily by a decrease in the value of services procured. Procurement of services decreased by 29% (US\$ 305.8 million), from US\$ 1071.3 million in 2024 to US\$ 765.5 million in 2025. Despite this decline, services remained the largest component of WHO procurement with 73% in 2025. The decrease in procurement of services was driven by major reductions across most of the big spending categories including Research and Normative work, Outsourcing Consulting and Audit Services, Outsourcing of Human Resources, Building Construction and Renovation and IT Development and Support, these being categories of procurement where non-essential spend could be deferred.

¹ Data for this report is drawn from WHO's enterprise resource planning (ERP) system (called the Global Management System or GSM). The Regional Office for the Americas does not use the same ERP system as the rest of WHO.

Procurement of goods also continued its downward trend, decreasing by 12% (US\$ 37.0 million) from US\$ 319.9 million in 2024 to US\$ 282.9 million in 2025. While the value of goods procured declined, the share of total procurement attributed to goods increased from 23% in 2024 to 27% in 2025, reflecting the sharper decline in the procurement of services.

At WHO, the procurement and supply of goods have historically been largely centralized and mostly handled by specialized procurement units and teams, particularly the Global Procurement and Logistics unit (GPL), including through catalogue procurement from established Long Term Agreements (LTAs), whereas the procurement of services is decentralized and mostly handled by requisitioning departments themselves. In 2024, WHO launched a plan to strengthen country offices and provide increased delegated authority to WHO Representatives to support faster decision-making (3). This includes greater autonomy over procurement activities, which may contribute to further localization of procurement over time.

Fig. 1. WHO procurement of goods and services, 2021–2025



2.2 Procurement by value

The total number of POs and total number of PO lines decreased in 2025 to a total of 47 098 POs and 130 583 PO lines. This represents a 19% decrease in total number of POs from 2024 (58 370 POs). The average value of POs decreased by 7% to US\$ 22 260 in 2025 (US\$ 23 834 in 2024), while the median decreased by 5% to US\$ 4498 in 2025 (US\$ 4720 in 2024).

It is important to distinguish between the average value of POs and the median value. The median is defined as the value separating the higher half from the lower half of all POs. The median is more representative of the “typical” value of a PO, as the average can be skewed by a small number of particularly high value POs.

Fig. 2 shows the number of POs approved across different value brackets. The distribution of number of POs across brackets remained broadly similar to 2024.

The majority of POs procured in 2025 continued to be in the smaller value ranges, with 80.5% of all POs at values below US\$ 20 000. The largest and smallest brackets in number of POs remained the same as in 2024, with the US\$ 500 to US\$ 2500 bracket being the largest (12 172 POs or 25.8% of total POs), and the over US\$ 1 000 000 bracket being the smallest (92 POs or 0.20% of total POs).

Fig. 2. Number of POs per value bracket

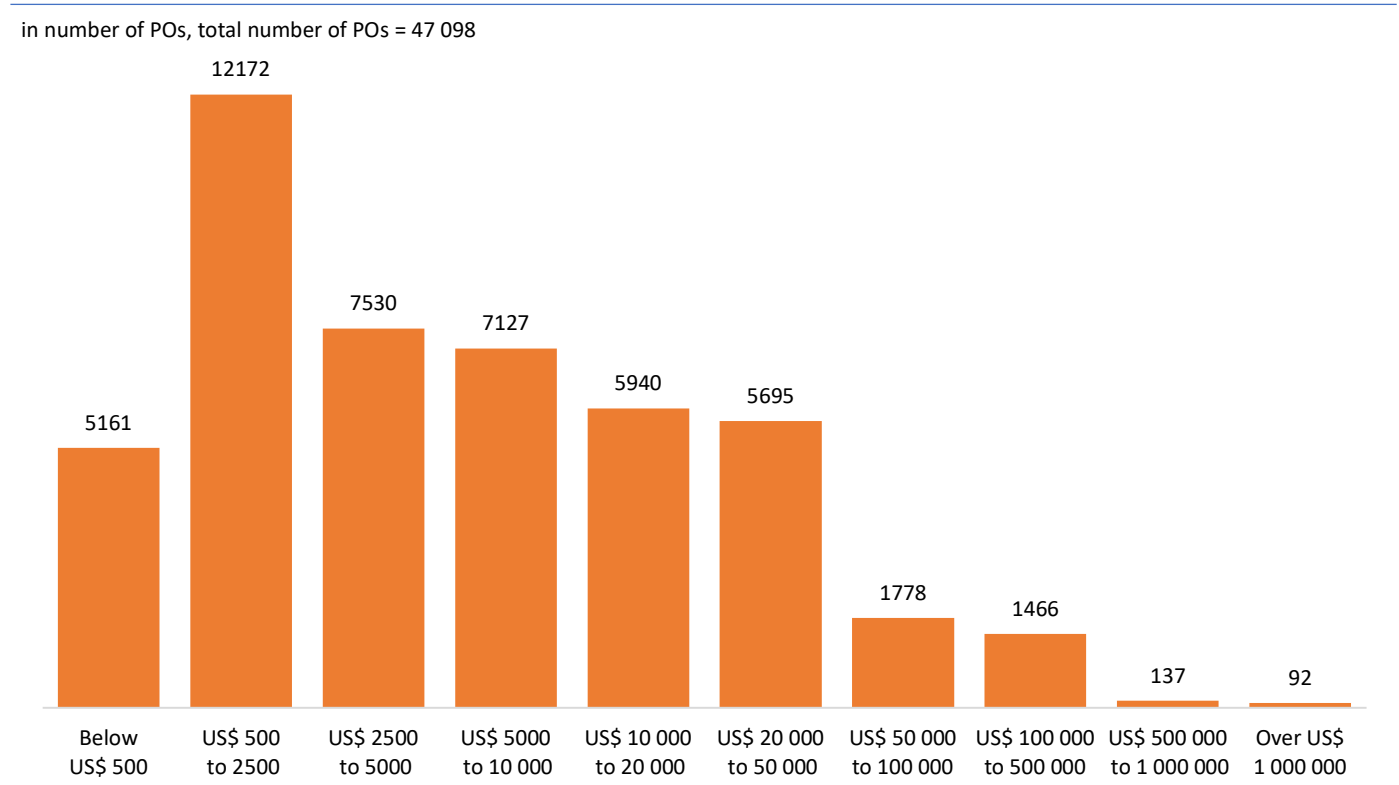
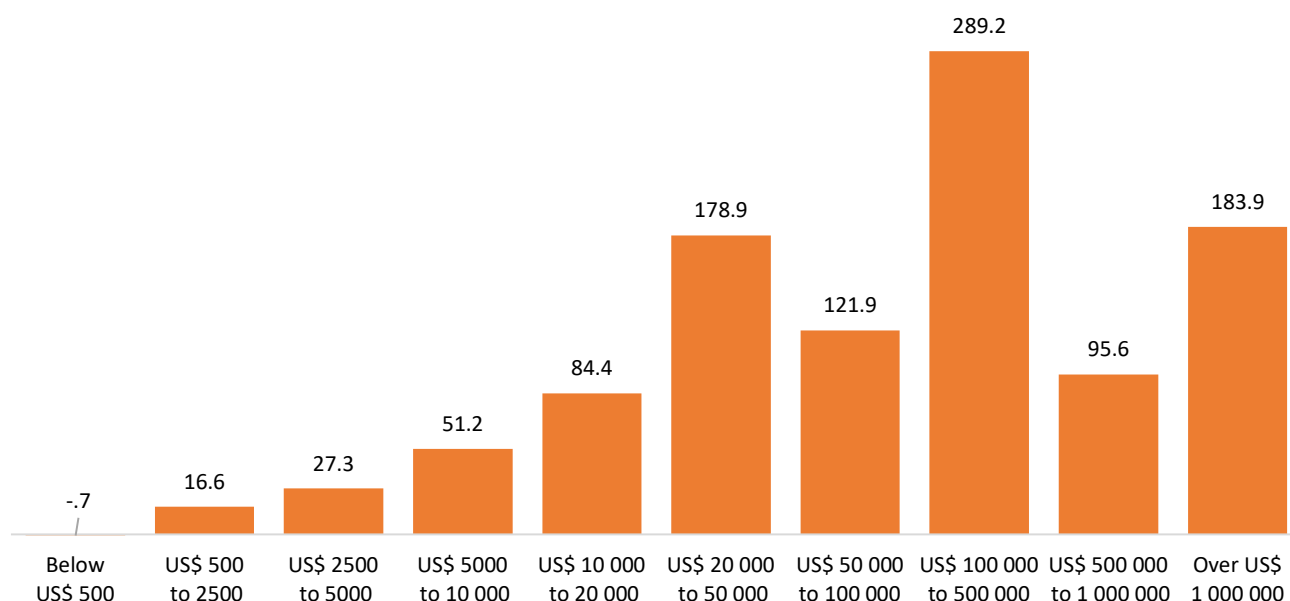


Fig. 3² provides an overview of the total value of POs within each value bracket. The majority of procurement spend in 2025 continued to come from a small number of high value POs, with 54.2% of all procurement spend coming from POs worth over US\$ 100 000.

² The negative value in the 'below US\$ 500' bracket reflects offsets applied to purchase order amendments. These offsets adjust for values reported in previous years to prevent double-reporting of purchase order values.

Fig. 3. Value of procurement per value bracket

in US\$ million, total value of procurement = US\$ 1048.4 million



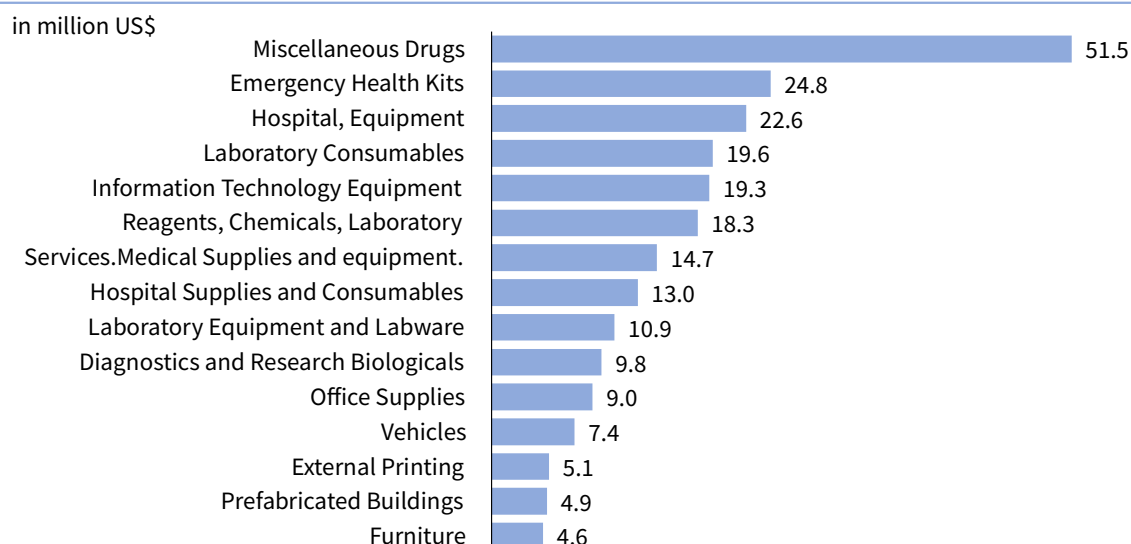
The largest bracket in terms of total value of POs in 2025 was the US\$ 100 000 to US\$ 500 000 bracket, accounting for US\$ 289.2 million (27.6% of total procurement value). The over US\$ 1 000 000 bracket, the smallest bracket in terms of number of POs, was the second largest in terms of value of procurement, accounting for US\$ 183.9 million (17.5% of total procurement value). The US\$ 500 to US\$ 2 500 bracket, the largest bracket in terms of number of POs, accounted for US\$ 16.6 million (1.6% of total procurement value). All POs under US\$ 20 000 accounted for 17.1% of the total value of procurement. Overall, procurement value remained concentrated in a relatively small number of high value POs.

2.3 Procurement by category

Fig. 4 and Fig. 5 show the top 15 categories of procurement expenditure for goods and services, respectively. The top five procurement categories for goods included Miscellaneous Drugs (4.9% of total procurement), Emergency Health Kits (2.4%), Hospital Equipment (2.2%), Laboratory Consumables (1.9%) and Information Technology Equipment (1.8%).

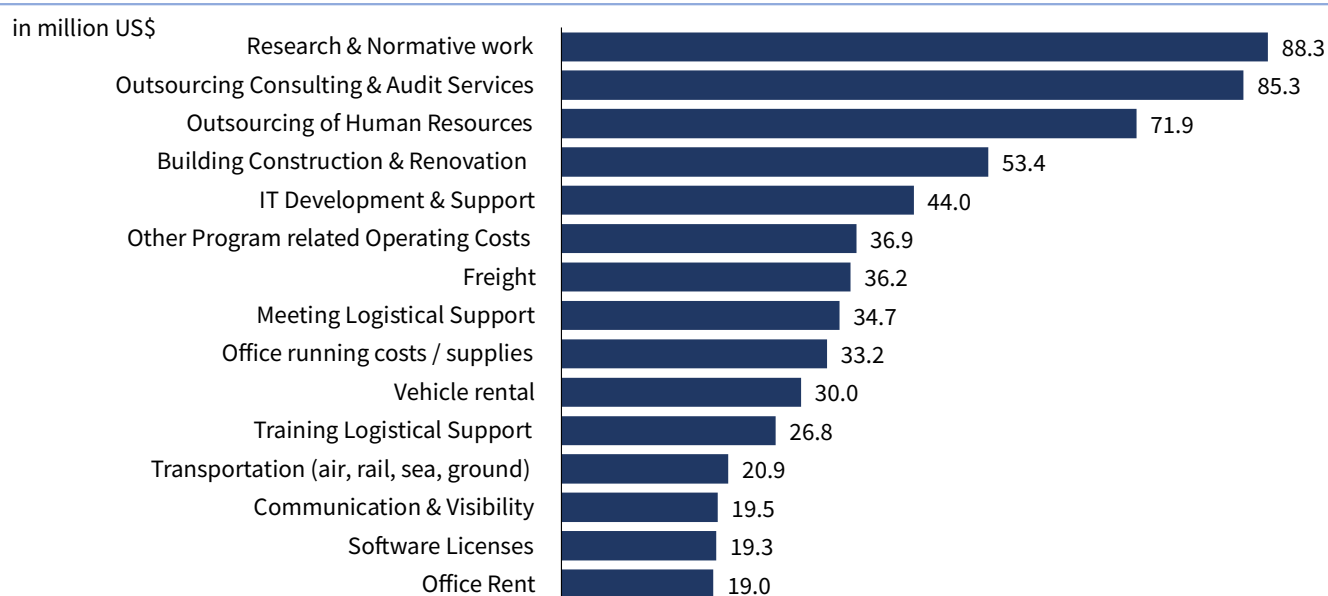
The top five goods categories remained the same as in 2024, although the composition of the top fifteen categories changed slightly, with Furniture and Prefabricated Buildings entering the top fifteen categories procured, while X-Ray Equipment and Accessories and Vehicles Equipment and Accessories dropped out. The decrease in procurement of goods in 2025 was driven largely by sharp declines in some of the highest-value categories, in particular Emergency Health Kits, which decreased by 49% from US\$ 48.8 million in 2024 to US\$ 24.8 million in 2025, and Hospital Equipment, which decreased by 34% from US\$ 34.1 million to US\$ 22.6 million. Miscellaneous Drugs remained the largest goods category with a more modest decrease of 11% from US\$ 58.1 million in 2024 to US\$ 51.5 million in 2025. At the same time, expenditure increased in a number of other goods categories, including Reagents, Chemicals, Laboratory, Vehicles and Information Technology Equipment.

Fig. 4. Largest procurement categories for goods



The top five categories for procurement of services remained the same and included Research and Normative work (8.4% of total procurement), Outsourcing Consulting and Audit Services (8.1%), Outsourcing of Human Resources (6.9%), Building Construction and Renovation (5.1%), and IT Development and Support (4.2%).

Fig. 5. Largest procurement categories for services



Expenditure declined across most of the top fifteen service categories in 2025, reflecting the overall reduction in procurement of services. Notably, there were considerable reductions in the top five categories. Research and Normative work decreased by 42% from US\$ 152.3 million in 2024 to US\$ 88.3 million in 2025, Outsourcing Consulting and Audit Services decreased by 41% from US\$ 143.7 million to US\$ 85.3 million and IT Development and Support services declined 36% to US\$ 44.0 million in 2025. Most notably, Building Construction and Renovation decreased by more than half from US\$ 113.8 million in 2024 to US\$ 53.4 million in 2025. A key reason for this has been the finalization of the WHO Academy in Lyon and construction to the WHO HQ which drove building and construction costs in 2024.

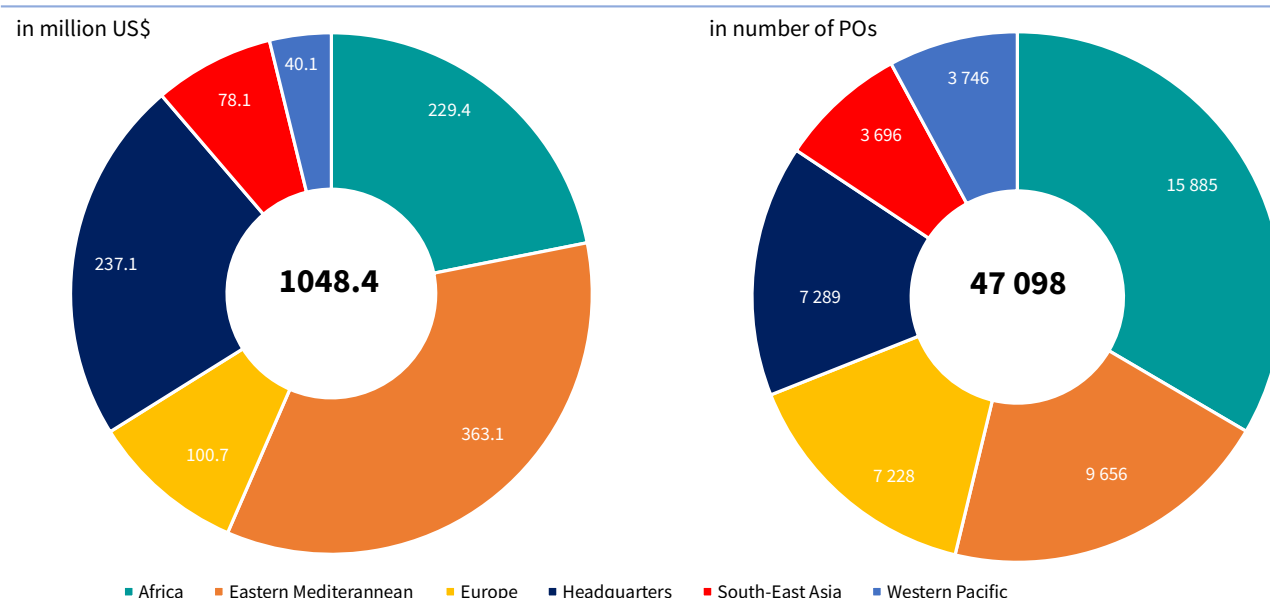
2.4 Procurement by WHO headquarters and regional offices

The largest share of procurement in 2025 came from the Regional Office for the Eastern Mediterranean with US\$ 363.1 million (34.6% of total procurement), followed by WHO headquarters with US\$ 237.1 million (22.6%), and the Regional Office for Africa in third place with US\$ 229.4 million (21.9%). Fig. 6 provides an overview of procurement for the regional offices and WHO headquarters (in total value and number of POs).

The order of the share of procurement across offices changed slightly since 2024, with the Regional Office for the Eastern Mediterranean moving above WHO headquarters as the office where the largest proportion of procurement occurred. Procurement declined across WHO headquarters and all regional offices in 2025. The sharpest decline was at WHO headquarters, where procurement decreased by 44% from US\$ 425.3 million in 2024 to US\$ 237.1 million in 2025. Procurement in the Regional Office for the Eastern Mediterranean decreased by 15% from US\$ 424.7 million to US\$ 363.1 million, while procurement in the Regional Office for Africa decreased by 10% from US\$ 256.2 million to US\$ 229.4 million. Europe, South-East Asia and the Western Pacific also recorded declines in procurement value.

The Regional Office for Africa remained the office with the largest number of POs (15 885 or 33.7% of all POs). Procurement in terms of number of POs declined across WHO headquarters and all regional offices, with the largest decrease recorded at WHO headquarters, where the number of POs fell by 36% from 11 439 in 2024 to 7 289 in 2025. The Regional Office for Europe also saw a marked decline in the number of POs, by 27% from 9 939 to 7 228. The reduction in number of POs across all regions and WHO headquarters is a reflection of the decision to introduce cost containment measures to mitigate the immediate impact of funding shortfalls.

Fig. 6. Procurement for WHO headquarters and regional offices in terms of value and number of POs



The relative decline in procurement at WHO headquarters in 2025 is in line with historic trends as shown in Fig. 7, which depicts procurement by WHO headquarters and regional offices over the period 2021–2025. Procurement at WHO headquarters has declined substantially from US\$ 720.5 million (43.5% of total procurement) in 2021 to US\$ 237.1 million (22.6%) in 2025. Procurement for other regions also declined compared to 2021 with the exception of the Regional Office for Africa which increased from US\$ 210.1 million (12.7%) in 2021 to US\$ 290.3 million in 2023

before declining in 2024 and 2025 to reach US\$ 229.4 million (21.9%) in 2025. The relative procurement share stayed constant in Europe, South-East Asia and the Western Pacific over time, but increased for the Regional Office of the Eastern Mediterranean from 26.7% in 2021 to 34.6% in 2025.

Fig. 7. Procurement by WHO headquarters and regional offices, 2021–2025

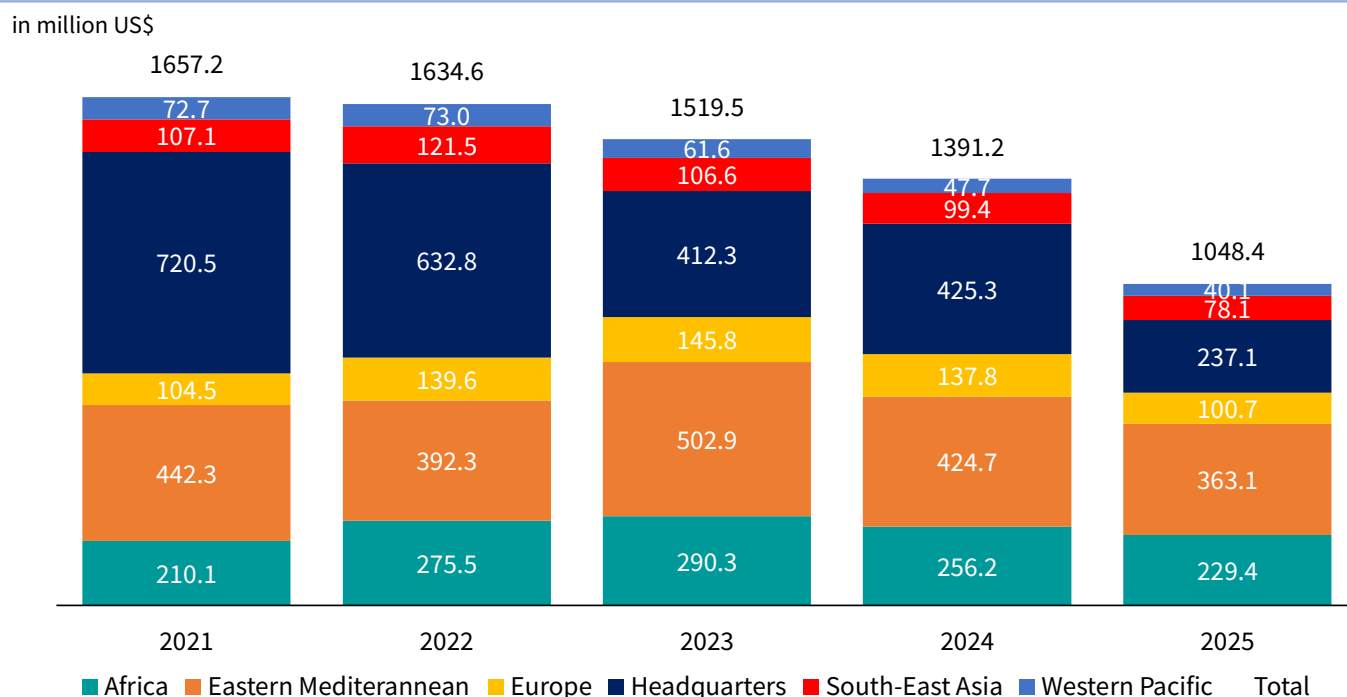
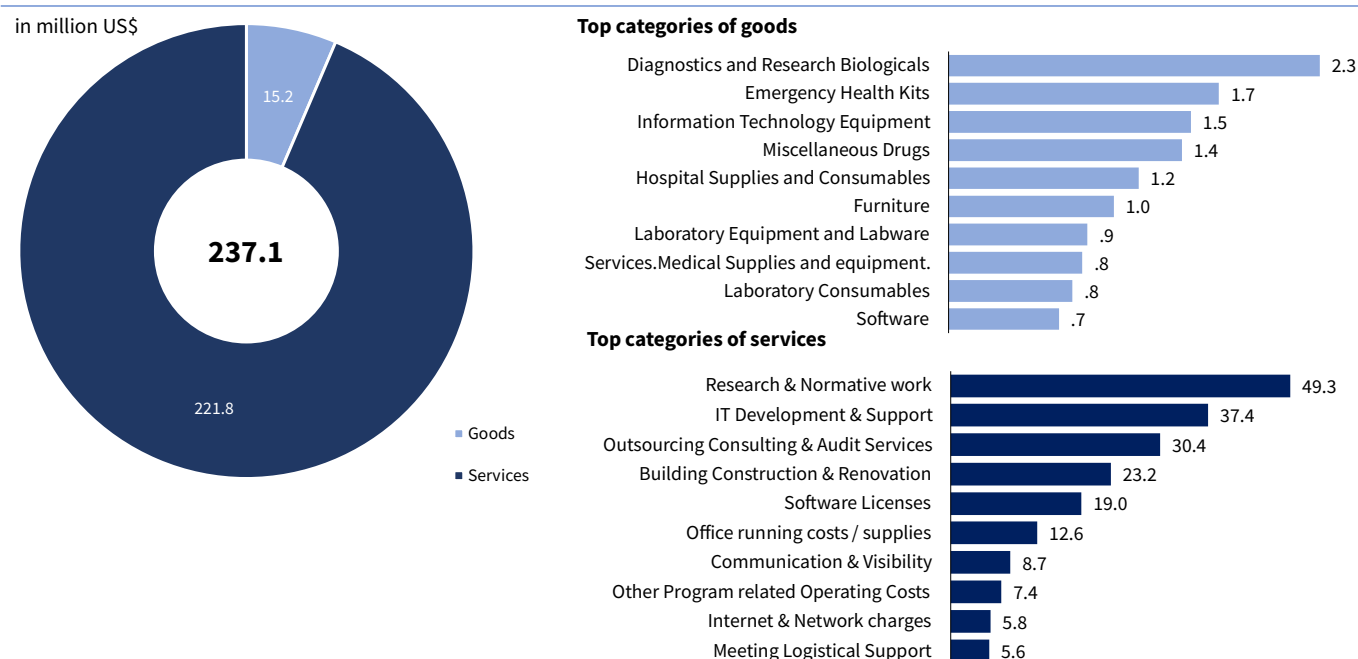


Fig. 8 shows the procurement profile for WHO headquarters with a breakdown across services and goods, and the top categories for each type of procurement. The WHO headquarters total procurement value in 2025 was US\$ 237.1 million, with US\$ 15.2 million for the procurement of goods (6.4%) and US\$ 221.8 million for the procurement of services (93.6%). The decline in WHO headquarters procurement was driven by decreases in both services and goods. Procurement of services decreased by 44% from US\$ 392.6 million in 2024 to US\$ 221.8 million in 2025, while procurement of goods decreased by 53% from US\$ 32.6 million to US\$ 15.2 million.

The largest procurement categories of goods for 2025 included Diagnostics and Research Biologicals, Emergency Health Kits, and Information Technology Equipment. The largest procurement categories of services procured by WHO headquarters in 2025 were Research and Normative work, IT Development and Support, and Outsourcing Consulting and Audit Services. Significant changes included a decrease in spending on Building Construction and Renovation from US\$ 82.7 million in 2024 to US\$ 23.2 million in 2025, alongside decreases in Research and Normative work from US\$ 89.5 million to US\$ 49.3 million and IT Development and Support from US\$ 61.3 million to US\$ 37.4 million. The regional offices' procurement profiles and details are included in Annex 1. Procurement summary by WHO regional office.

Fig. 8. Procurement overview for WHO headquarters (see details for regional offices in Annex 1)



2.5 Procurement by supplier and supplier country or territory

WHO procured goods and services from a total of 12 935 suppliers in 2025, which was a 21% decrease from 16 447 suppliers in 2024.

As in 2024, the top fifteen suppliers in 2025 were predominantly providers of services rather than goods, with ten of the top fifteen suppliers falling into this group. UNOPS became the top supplier in 2025 with US\$ 36.3 million, followed by Imres B.V. with US\$ 18.9 million, UNDP with US\$ 17.8 million, and CTG Global Managed Services DMCC with US\$ 15.9 million. Compared with the 2024 top fifteen supplier list, CTG Global Managed Services DMCC, Apleona HSG SA, XEAM Ventures Private Limited and AMEX entered the top fifteen in 2025, while Missionpharma A/S, Company Toleran LLC, Workday Limited and Videlio dropped out. Fig. 9 shows the top 15 suppliers and their respective procurement volume by goods and services.

Fig. 9. Largest suppliers

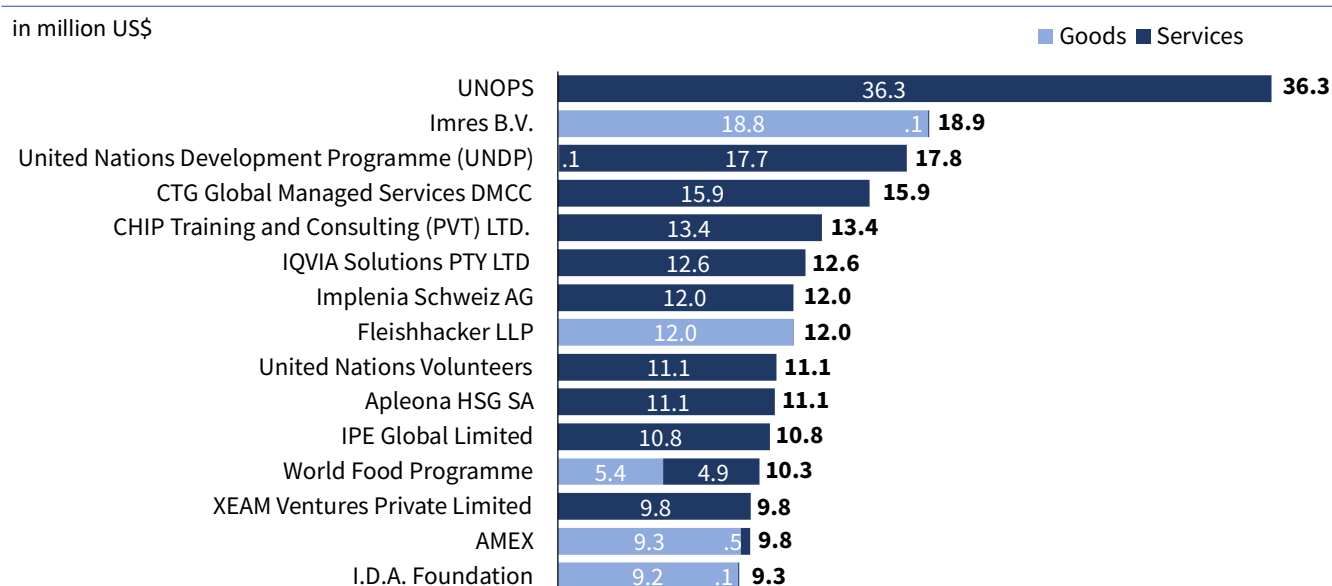
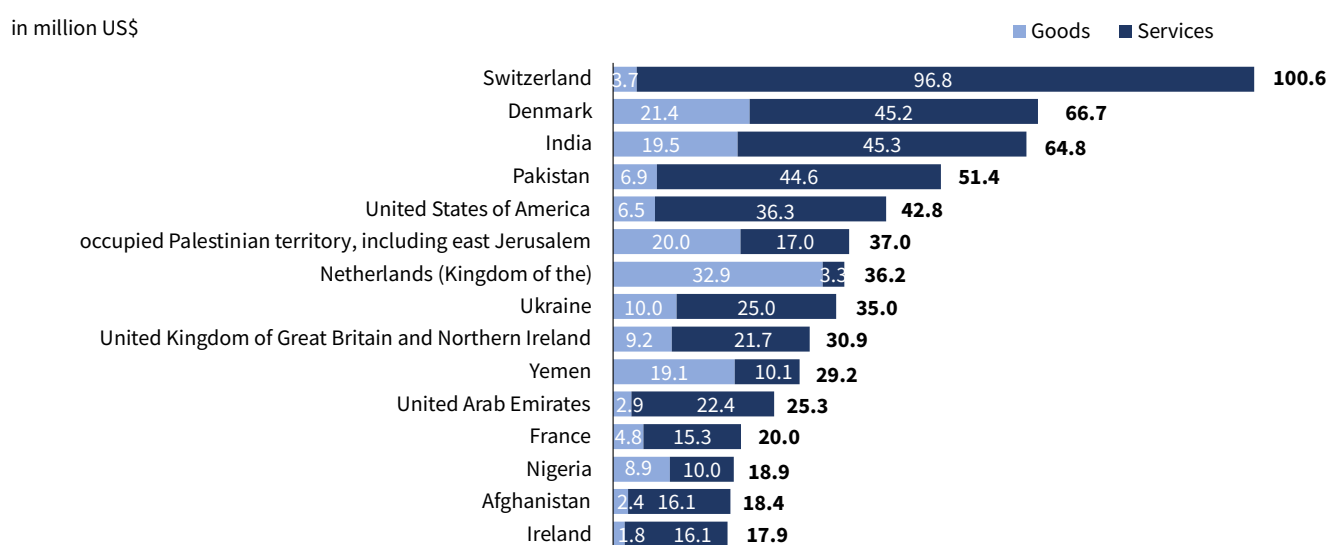


Fig. 10 shows where WHO's top suppliers were located in 2025. Switzerland, Denmark, India, Pakistan and the United States of America made up the top five supplier countries, and accounted for US\$ 326.3 million (31% of overall procurement) in 2025. Compared with the 2024 top fifteen supplier country list, Afghanistan, Nigeria and Yemen entered the top fifteen in 2025, while Italy, Germany and South Africa dropped out.

Among the supplier countries included in the 2025 top fifteen, procurement increased in Yemen by 31% from US\$ 22.3 million in 2024 to US\$ 29.2 million in 2025, and in Nigeria by 26% from US\$ 15.0 million to US\$ 18.9 million. By contrast, procurement declined in most of the other countries in the 2025 top fifteen, including France, which fell by 56% from US\$ 45.6 million to US\$ 20.0 million, Switzerland, which fell by 38% from US\$ 162.0 million to US\$ 100.6 million, and Pakistan, which fell by 37% from US\$ 81.9 million to US\$ 51.4 million.

Fig. 10. Top supplier countries or territories



2.6 Procurement from catalogues

WHO establishes and uses long-term agreements (LTAs) and catalogues to enable the efficient processing and procurement of frequently requested goods and services. A catalogue is a list of specific goods, accessible online on the WHO intranet, with standardized specifications across all WHO offices for which LTAs have been established with suppliers through a competitive procurement process. Once the catalogue is set up, the Global Procurement and Logistics unit can issue POs directly to suppliers, based on requisitions from requesting offices and internal clients.

Fig. 11 shows procurement of goods from catalogues over the period 2022–2025. Over this period, both total goods procurement and the value of goods procured from catalogues declined. Goods procured from catalogues fell from US\$ 136.8 million in 2022 to US\$ 62.5 million in 2025. However, the proportion of goods procured from catalogues stayed relatively stable ranging between 21% to 25% across all years.

Fig. 11. Procurement of goods from catalogues, 2022–2025

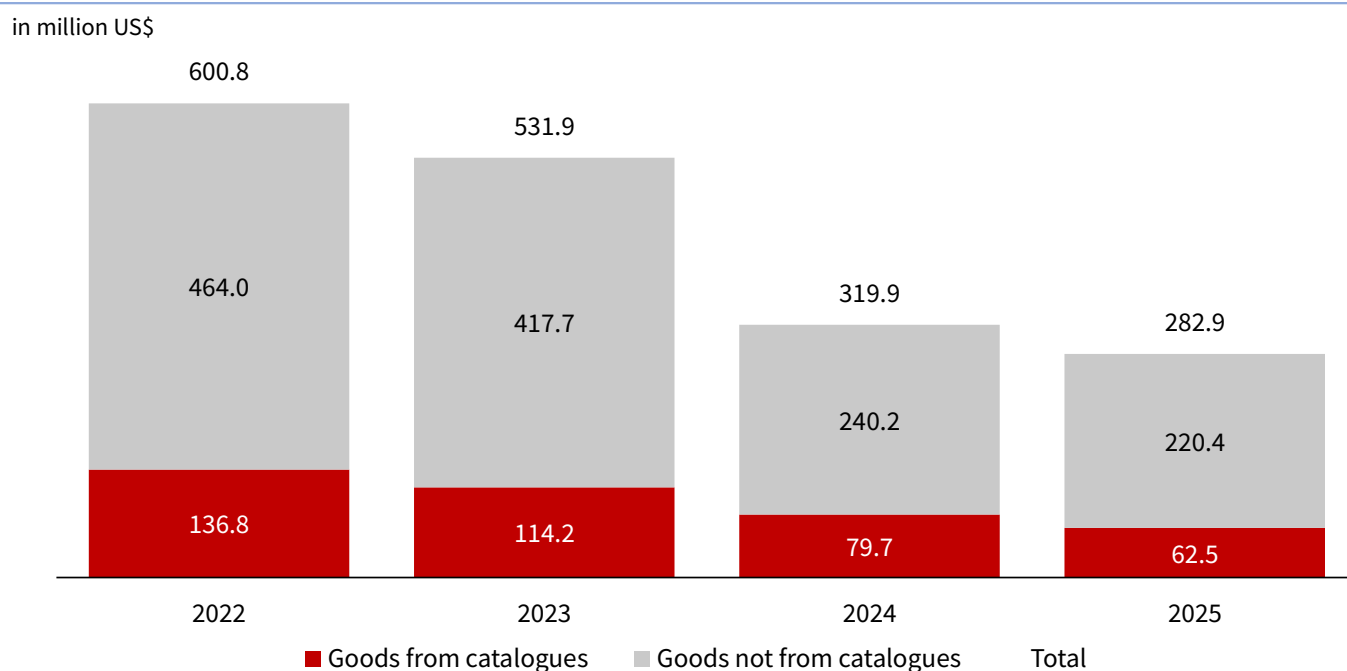
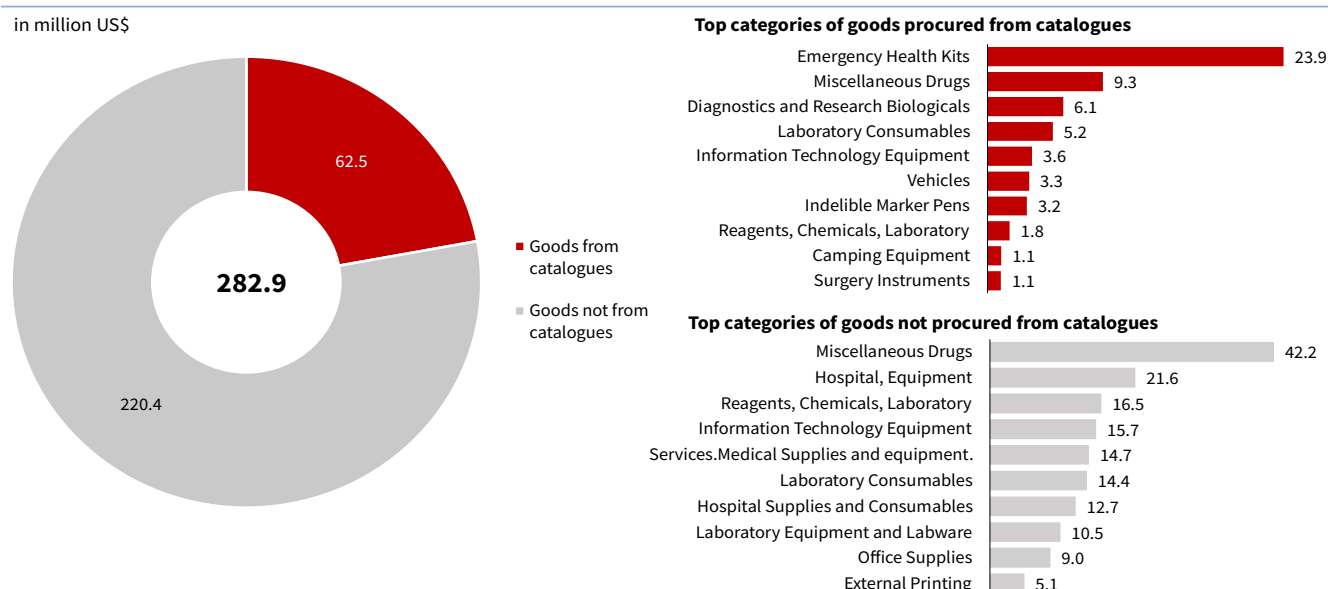


Fig. 12 shows the value and categories of goods that were procured through catalogues in 2025. Of the total US\$ 282.9 million spent on goods in 2025, US\$ 62.5 million was procured from catalogues. While this represents a decline of 22% from US\$ 79.7 million in 2024, this is reflective of an overall decline in the procurement of goods, and the proportion of goods procured from catalogues decreased slightly from 25% in 2024 to 22% in 2025.

The largest categories of goods procured through catalogues remained consistent compared with 2024, with Emergency Health Kits, Miscellaneous Drugs and Diagnostics and Research Biologicals in the top three. Emergency Health Kits remained the largest category procured from catalogues, although procurement declined by 39% from US\$ 39.2 million in 2024 to US\$ 23.9 million in 2025. Miscellaneous Drugs procured from catalogues increased by 18% from US\$ 7.9 million to US\$ 9.3 million. Among goods not procured from catalogues, Miscellaneous Drugs remained the largest category at US\$ 42.2 million, followed by Hospital Equipment at US\$ 21.6 million and Reagents, Chemicals, Laboratory at US\$ 16.5 million.

Fig. 12. Procurement of goods from catalogues



2.7 Regional procurement

Regional procurement refers to the procurement of goods and services from suppliers located in the same geographical region as the procuring office. Fig. 13 shows the proportion of regional procurement versus procurement of goods and services from suppliers located outside the region of the procuring office. In 2025, the Regional Office for Europe had the largest share of regional procurement at 93%, followed by the Regional Offices for South-East Asia (86%), the Western Pacific (76%), Africa (75%) and the Eastern Mediterranean (65%).

Fig. 13. Proportion of regional procurement

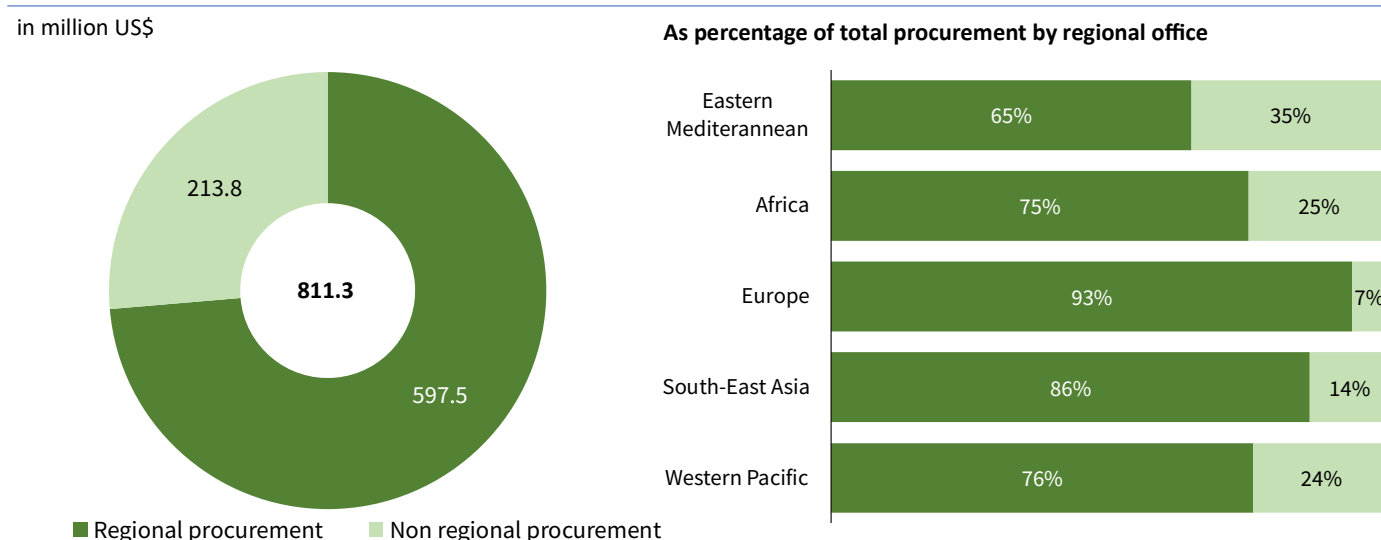


Fig. 14 shows regional versus non-regional procurement over time in terms of value. Since 2023, the value of procurement conducted regionally has remained consistently higher than the value procured outside the region, although both have declined in recent years. Regional procurement increased from US\$ 724.0 million in 2022 to US\$ 749.4 million in 2023, before declining in 2024 and 2025 to US\$ 724.4 million and US\$ 597.5 million respectively. Non-regional procurement, by contrast, declined much more sharply over the same period, falling from US\$ 910.6 million in 2022 to

US\$ 357.8 million in 2023, US\$ 241.5 million in 2024 and US\$ 213.8 million in 2025. Consequently, the share of procurement conducted regionally rose steadily from 38.3% in 2021 to 75.0% in 2024, before stabilising around that level in 2025, when regional procurement accounted for 73.6% of total procurement by regional offices.

Fig. 14. Regional procurement, 2021–2025

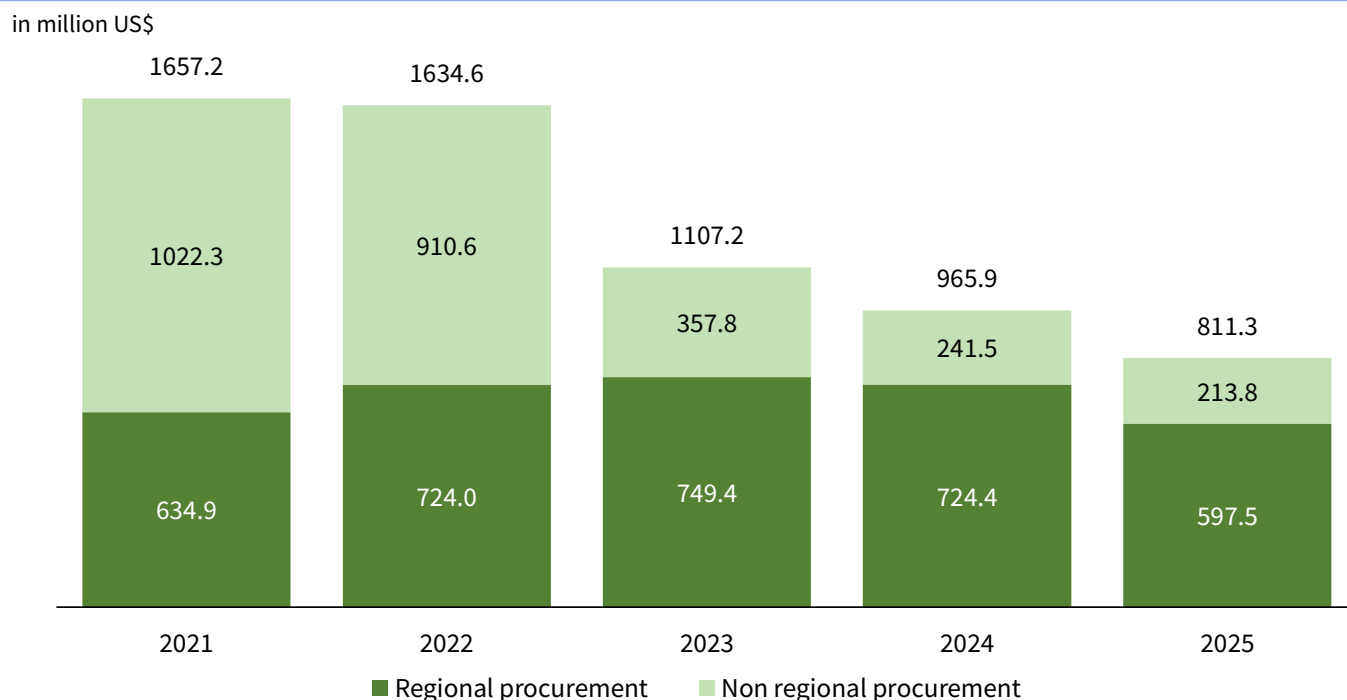


Fig. 15 shows the proportion of regional procurement of goods and services by Regional Office. Across all Regional Offices in 2025, the proportion of services procured regionally was higher than the proportion of goods procured regionally. Overall, 85% of services procured by Regional Offices were procured regionally, compared with 51% of goods. The Regional Office for Europe stands out as having particularly high proportions of regional procurement for both goods (85%) and services (96%). By contrast, the proportion of goods procured regionally was below half in the Regional Offices for the Eastern Mediterranean (48%) and Africa (37%).

Fig. 15. Proportion of regional procurement, goods vs. services

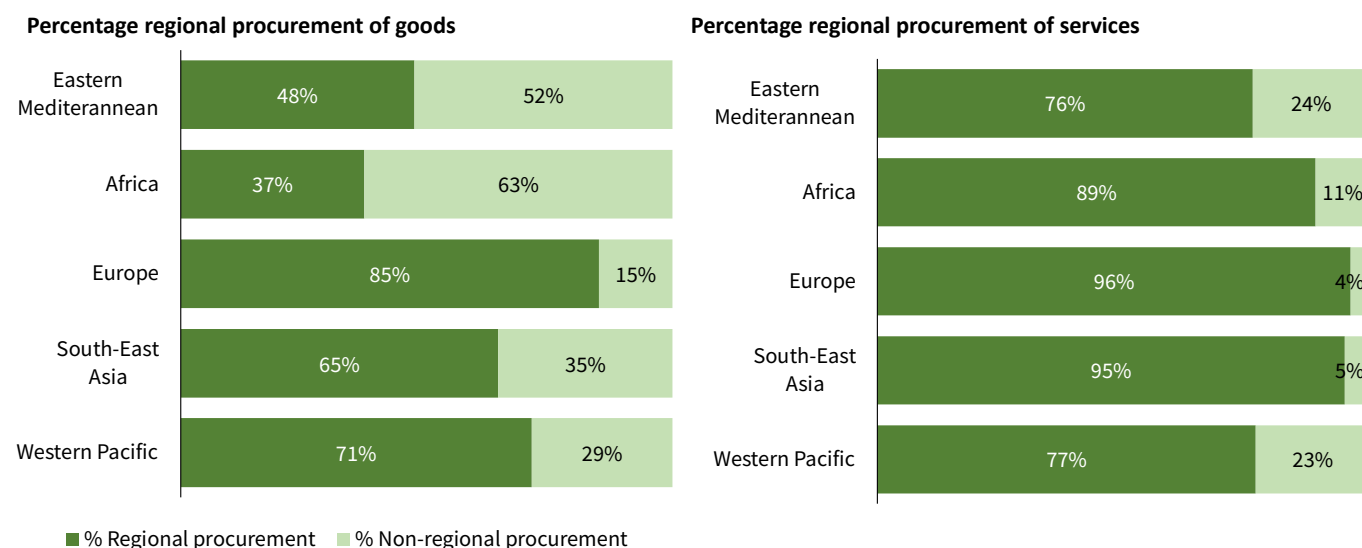
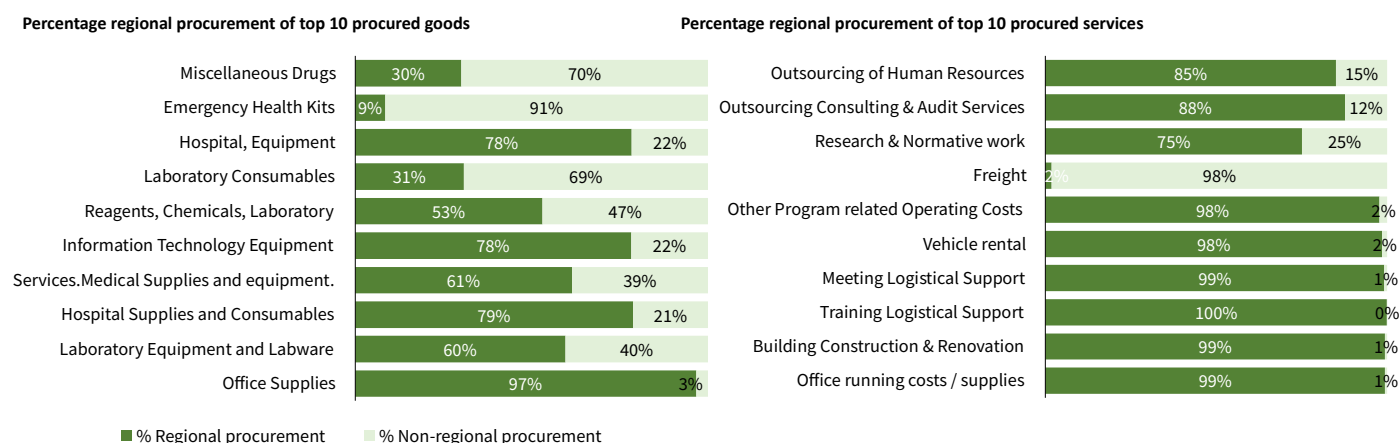


Fig. 16 depicts the proportion of regional procurement for the top 10 goods and services categories across all five Regional Offices. Among the top ten goods categories, the proportion of regional procurement was generally lower and more variable than for services. This was particularly the case for the two largest goods categories, with 30% of Miscellaneous Drugs and 9% of Emergency Health Kits procured regionally. By contrast, Office Supplies had a very high share of regional procurement at 97%, while Hospital Supplies and Consumables, Hospital Equipment and Information Technology Equipment also had relatively high regional shares, at 79%, 78% and 78% respectively.

The proportion of regional procurement remained high across most of the top ten categories of services. Training Logistical Support was entirely procured regionally in 2025, while Building Construction and Renovation, Office running costs / supplies and Meeting Logistical Support all had regional shares of 99%. The main exception was Freight, of which only 2% was procured regionally. This category of spend being largely managed through global LTAs. Research and Normative work was also less regionally procured than most service categories, with 75% procured regionally.

Fig. 16. Proportion of regional procurement (all Regional Offices), for top 10 goods and services



2.8 Collaborative procurement

Collaborative procurement with other UN organizations plays an important role within the procurement approach of WHO. Collaborative procurement is defined as a procurement arrangement in which several UN organizations combine their efforts to undertake procurement in cooperation or share the outcome of a procurement process, thereby achieving benefits for the group in its entirety. The objective of collaborative procurement is to achieve reduced price or better service through economies of scale and to reduce inefficiency and duplication across UN organizations.

In 2025, a total of US\$ 117.8 million was procured collaboratively with other UN organizations, around 11.2% of total procurement (Fig. 17). This represents a slight decrease compared with 2024, when 11.6% of all goods and services were procured collaboratively. The decline in overall collaborative procurement reflects decreases in both collaboratively procured services and goods. The value of collaboratively procured services decreased from US\$ 129.5 million in 2024 to US\$ 93.7 million in 2025, while collaboratively procured goods decreased from US\$ 31.5 million to US\$ 24.1 million. As a proportion of total services procured, collaborative procurement remained broadly stable at 12.2% in 2025 compared with 12.1% in 2024. By contrast, the proportion of collaboratively procured goods declined from 9.8% in 2024 to 8.5% in 2025.

Goods categories with relatively high collaborative procurement included Services, Medical Supplies and Equipment, Information Technology Equipment, and Hospital Supplies and Consumables. Fig. 17 shows collaborative procurement by goods and services and the top categories procured.

Fig. 17. Collaborative procurement

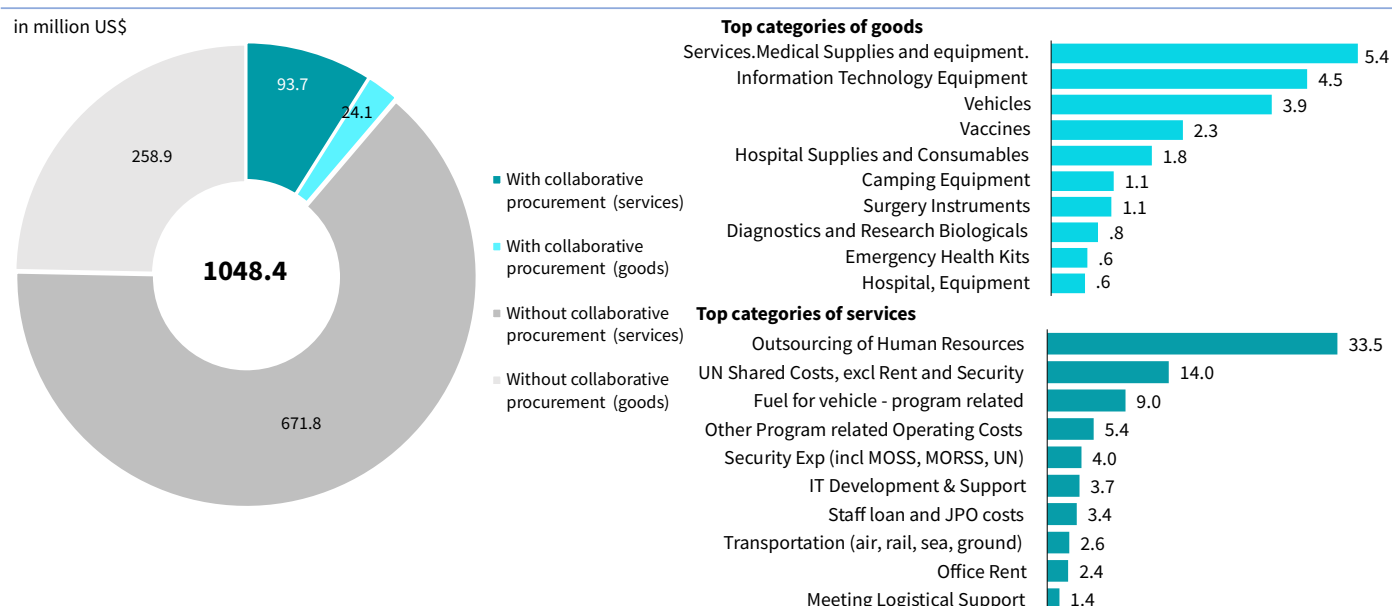


Fig. 18 shows the proportion of collaborative versus non-collaborative procurement amongst the top ten categories of goods and services procured by WHO.

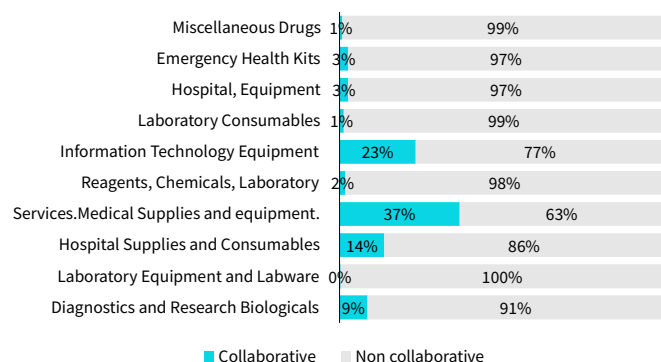
Collaborative procurement across the top three categories of goods procured remained low in 2025, at 1% of total procurement for Miscellaneous Drugs, 3% for Emergency Health Kits, and 3% for Hospital Equipment. Amongst the top ten categories of goods procured, the categories with the highest proportion of collaborative procurement were Services for Medical Supplies and

Equipment (37%) and Information Technology Equipment (23%). Hospital Supplies and Consumables also had a relatively higher collaborative share at 14%.

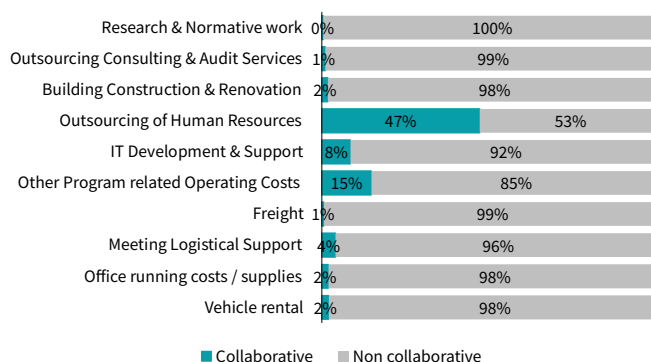
Across the top three categories of services procured, collaborative procurement also remained very low, at 0% of total procurement for Research and Normative work, 1% for Outsourcing Consulting and Audit Services, and 2% for Building Construction and Renovation. Among the top ten categories of services procured, Outsourcing of Human Resources had the highest proportion of collaborative procurement at 47%, followed by Other Program related Operating Costs at 15% and IT Development and Support at 8%.

Fig. 18. Collaborative procurement, for top 10 goods and services

Percentage collaborative procurement of top 10 procured goods

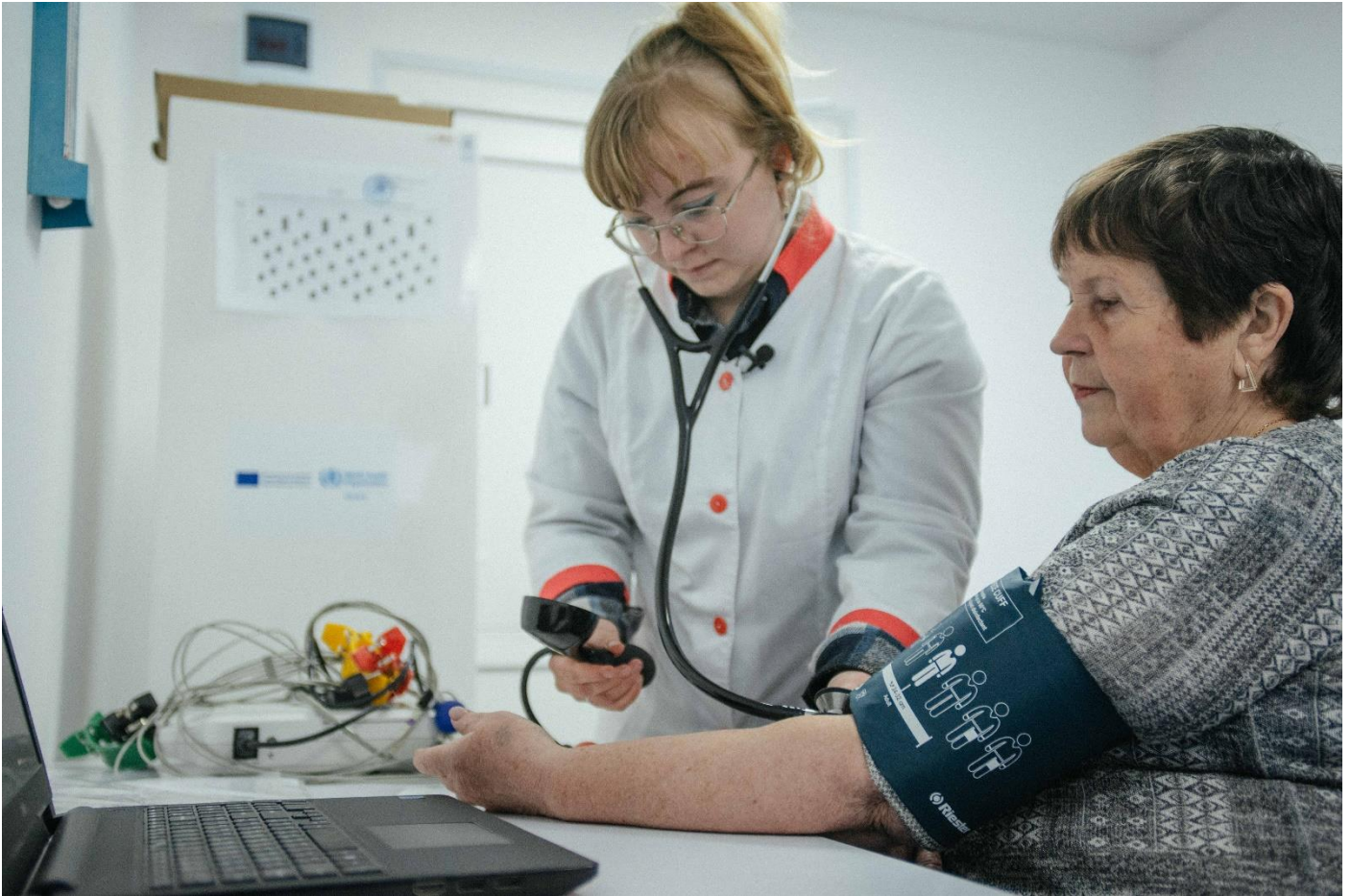


Percentage collaborative procurement of top 10 procured services



PART 3

Key initiatives and highlights



Modular clinic near the frontline in Kharkiv region - © WHO.

A woman visiting a doctor in a modular clinic near the frontline, established by WHO, Vysokopillya village, Kharkiv region, Ukraine, April 2025.

In 2025, WHO procurement operated in a constrained and volatile financial environment, marked by funding uncertainty, cost containment requirements and the closure of the 2024–2025 biennium. Despite a significant reduction in overall procurement volumes compared to previous years, the function continued to deliver timely, compliant and value for money procurement in support of priority programmes, health emergencies and country operations across all regions. Procurement activity in 2025 reflected a deliberate shift toward prioritization, consolidation and strengthened controls, while maintaining service continuity and operational readiness worldwide.

Procurement volumes in 2025 declined across both goods and services, reflecting early and sustained cost-containment measures and tighter prioritization of non-essential expenditure. The Organization implemented a set of measures, beginning in the first quarter of 2025, to align procurement demand with available resources while protecting priority health programmes and emergency operations. These measures included temporary freezes on non-critical procurement categories, tighter prioritization of requirements, increased use of existing long-term agreements and UN collaborative contracts, consolidation of demand, restrictions on travel and in-person meetings, and the deferral of discretionary investments and non-essential renovations, alongside strengthened supplier negotiations and value-for-money reviews.

Implemented under established governance and oversight arrangements, these actions aimed to mitigate financial risk, preserve liquidity, and demonstrate responsible use of Member State and donor resources, while maintaining compliance with WHO procurement rules and ensuring that procurement decisions remained transparent, documented, and defensible to oversight bodies.

In this constrained context, across 2025, WHO procurement continued to support a range of high-value and strategically significant activities, including emergency health kits and diagnostics for fragile and crisis-affected settings, large-scale laboratory and equipment procurements linked to pandemic preparedness and the Pandemic Fund, and continued investment in digital and IT services critical to organizational transformation. At the same time, procurement teams intensified negotiations with long-term agreement holders, consolidated demand across programmes and regions, and strengthened category management approaches to achieve cost savings and mitigate supply risks. Regional Offices increasingly accounted for a larger share of procurement activity, reinforcing the trend toward regional and country-level execution supported by global frameworks, long-term agreements and catalogue procurement.

WHO procurement and supply chain initiatives were also recognized externally, including through the UN Procurement Awards, highlighting innovation in pandemic preparedness for a project led by the Berlin Hub team, and in digitization and capacity development for a project led by the Kuala Lumpur based logistics team in conjunction with the Business Management Systems team.

In 2025, WHO advanced important policy and control enhancements, including approval of new global guidance on warehousing and inventory management and IT procurement, both issued as policy updates in Quarter one of 2026, consolidating previously fragmented practices into a single organizational framework.

Procurement in 2025 faced multiple external and internal challenges, including funding shortfalls, late programme requests, logistics disruptions, freight volatility and system limitations during ongoing business management system transformation. Procurement teams responded by reprioritising workloads, supporting country offices with planning and consolidation, strengthening supplier engagement, and leveraging digital dashboards and data to improve visibility and

coordination. Through flexibility, collaboration and professional judgment, the procurement function continued to deliver value across WHO, balancing financial discipline with the imperative to support health outcomes globally.

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Annexes



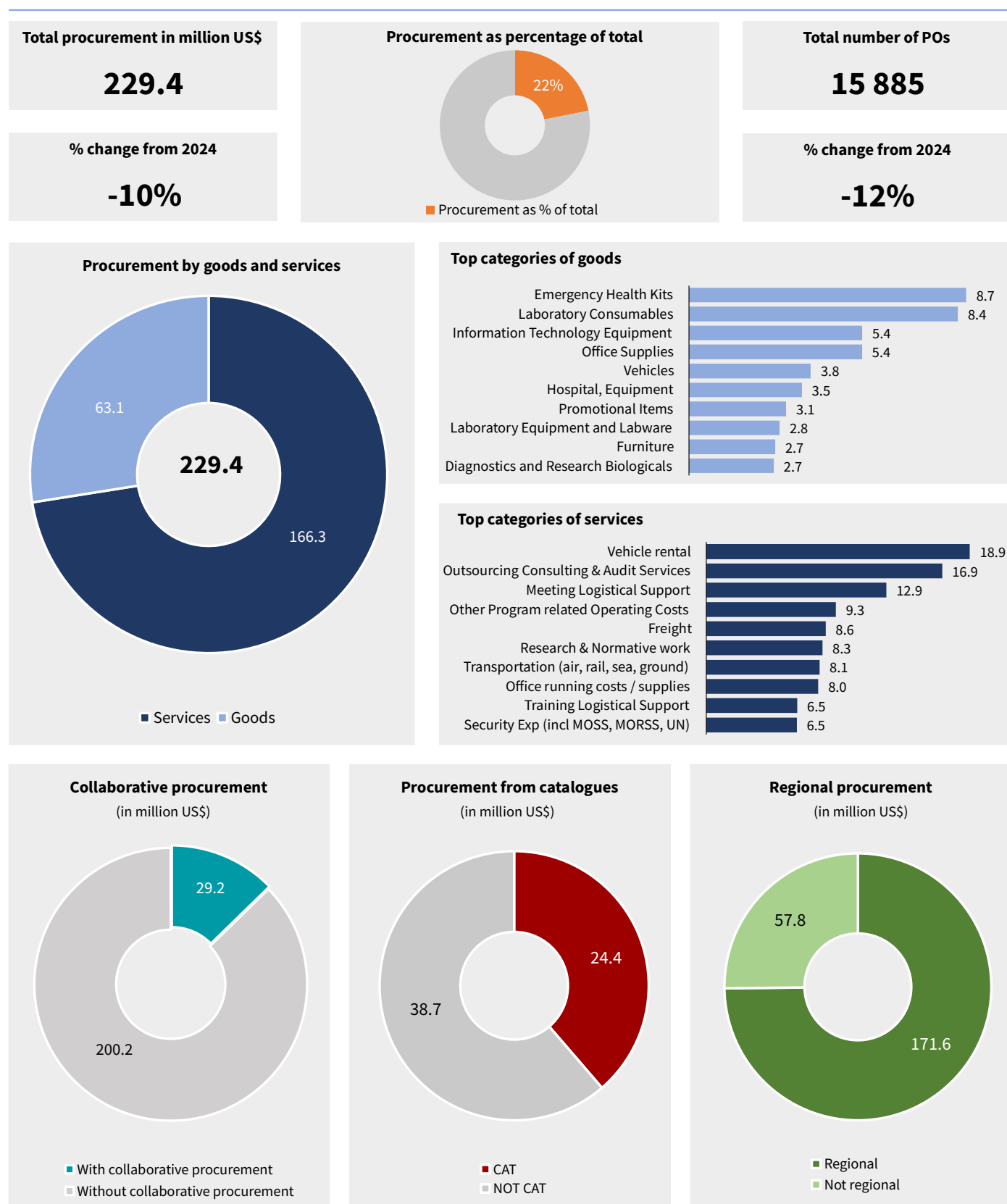
Over 35 metric tonnes of WHO medical supplies arrived in Kabul to support earthquake survivors in eastern Afghanistan - 8 September 2025 - © WHO / Zakarya Safari.

A new consignment of more than 35 metric tonnes of life-saving medical supplies landed in Kabul to enhance the World Health Organization's (WHO) emergency response to the devastating 6.0 magnitude earthquake that struck eastern Afghanistan on 31 August. With this shipment, WHO prepositioned and delivered nearly 80 metric tonnes of emergency health supplies to the country since the disaster.

The cargo, mobilized through WHO's logistics hub in Dubai, included trauma and emergency surgery kits, primary health care kits, noncommunicable disease kits and essential medicines. These supplies were dispatched to health facilities and mobile health teams in the hardest-hit areas, following the needs assessments, enabling doctors and nurses to provide urgent treatment, perform emergency surgeries and help prevent the spread of disease.

Annex 1. Procurement summary by WHO regional office³

Fig. A1.1. Procurement at the Regional Office for Africa



³ Data for this report is drawn from WHO's enterprise resource planning (ERP) system (called the Global Management System or GSM). The Regional Office for the Americas does not use the same ERP system as the rest of WHO.

Fig. A1.2. Procurement at the Regional Office for the Eastern Mediterranean

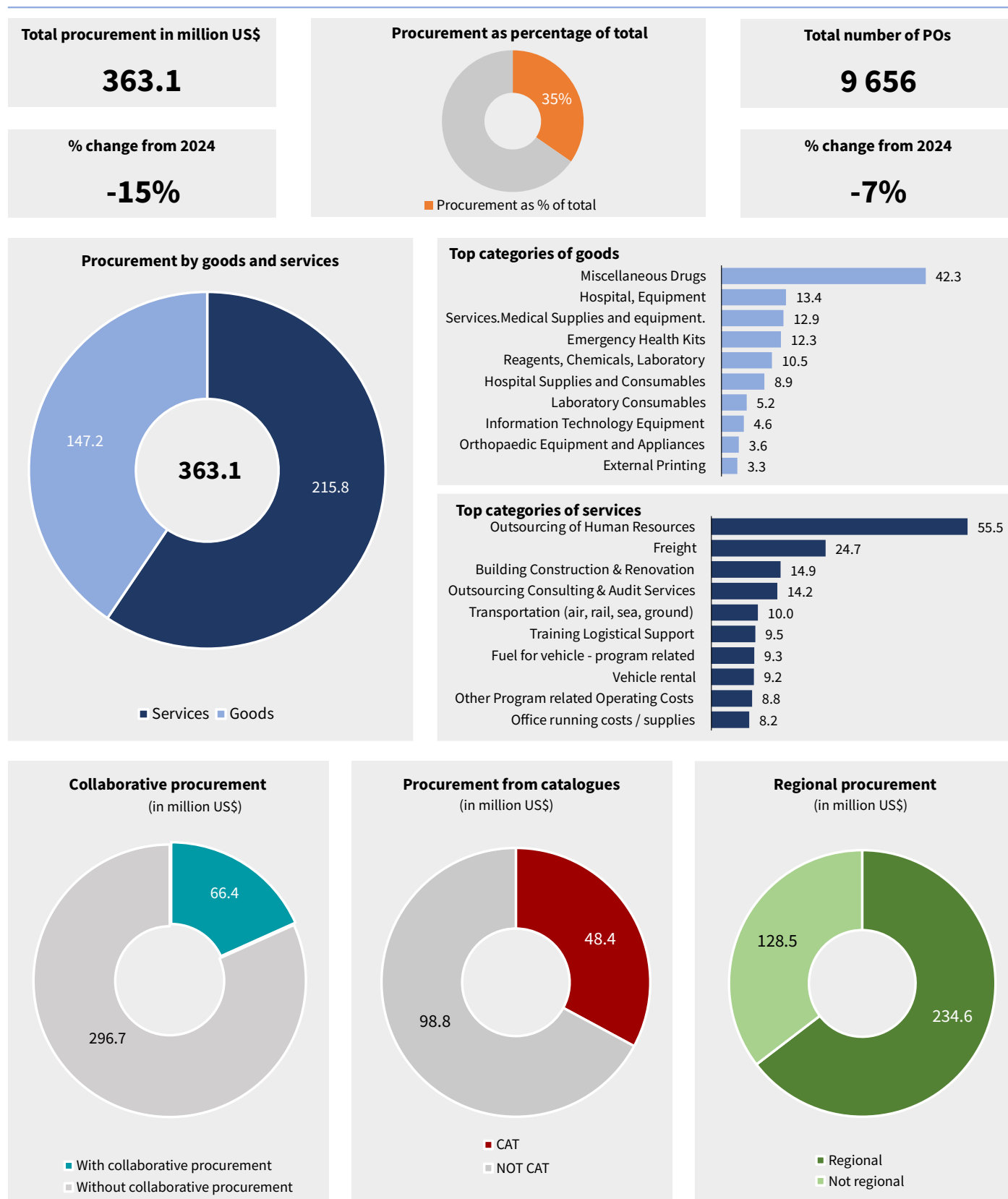


Fig. A1.3. Procurement at the Regional Office for Europe

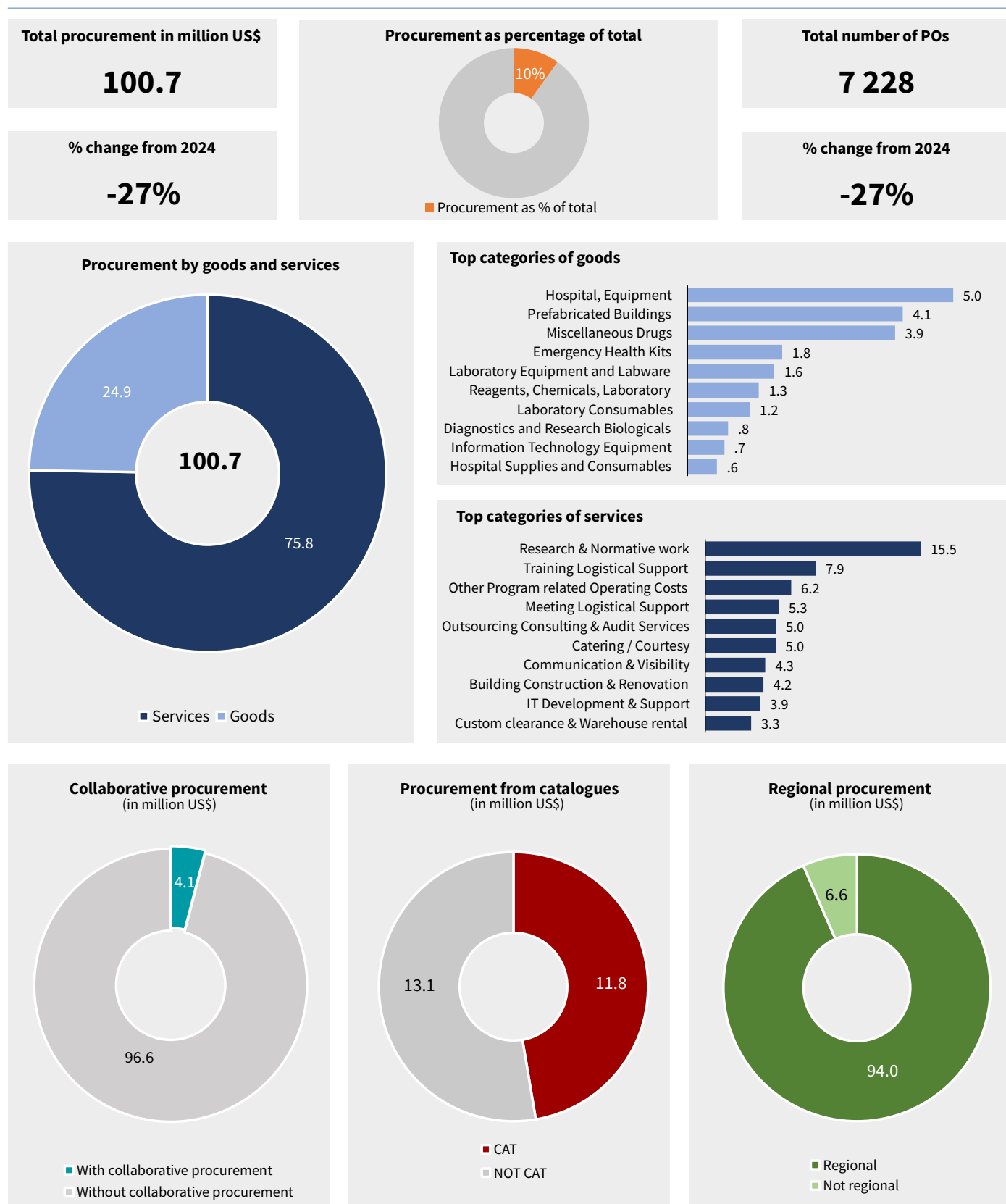


Fig. A1.4. Procurement at the Regional Office for South-East Asia

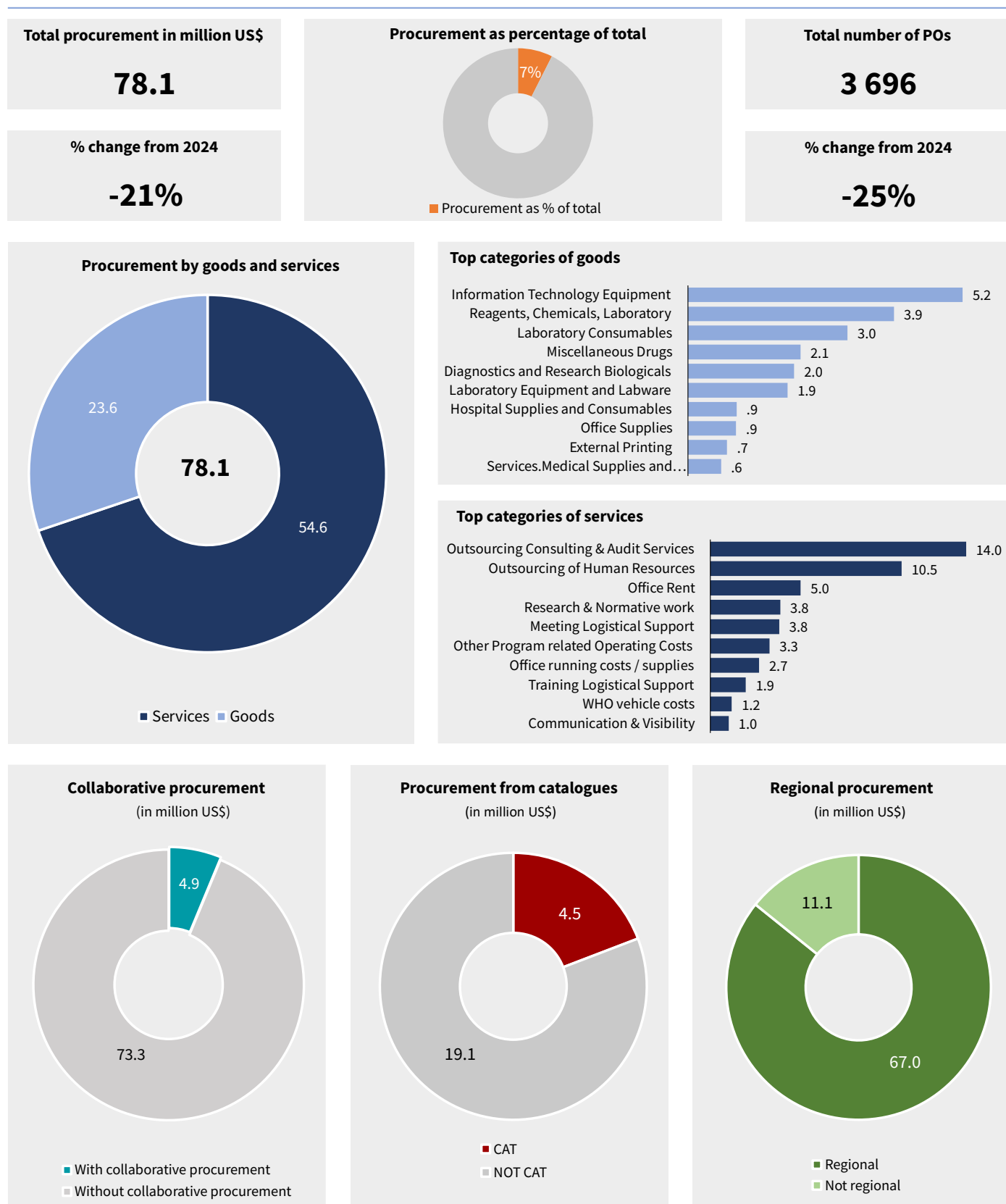
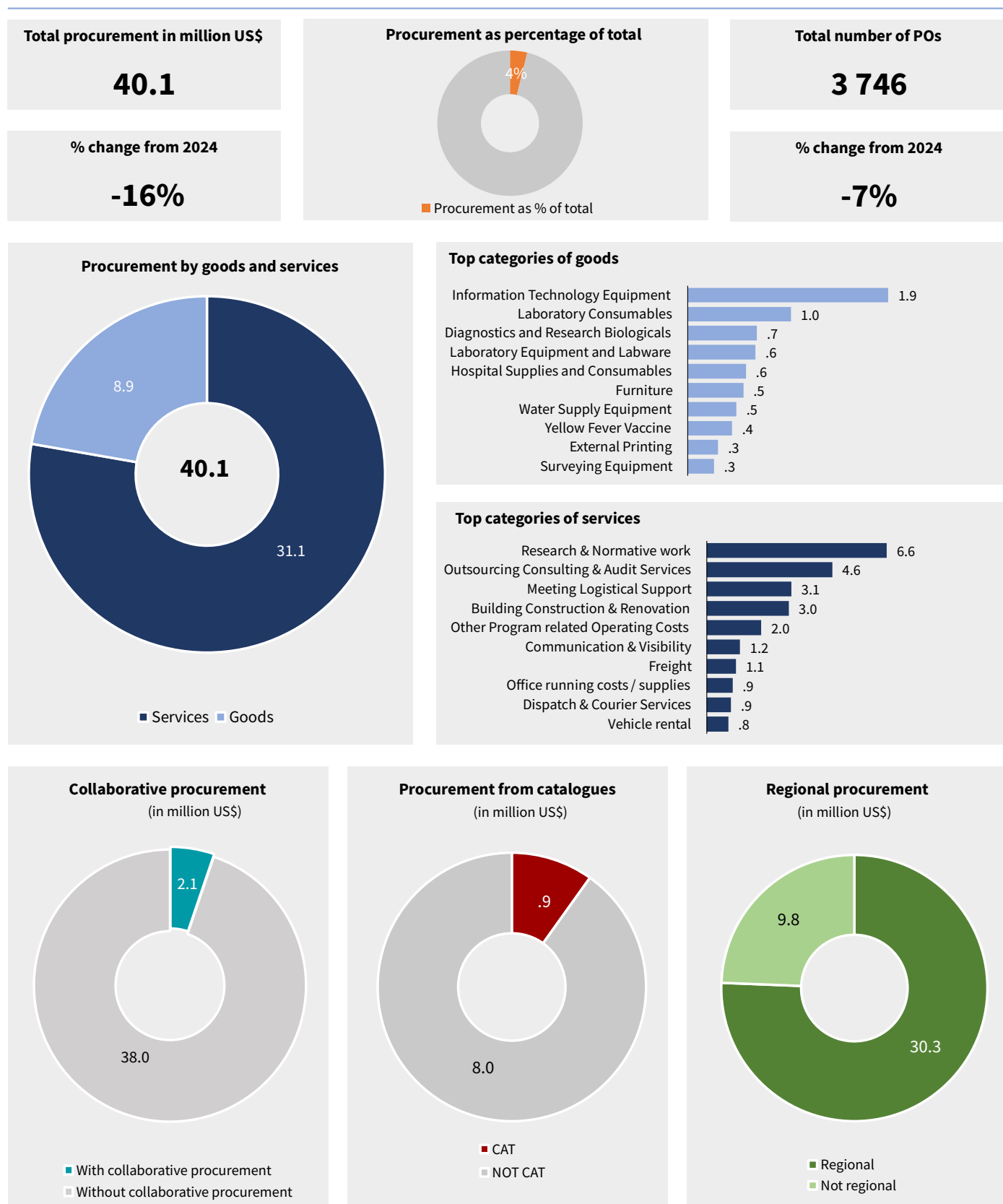


Fig. A1.5. Procurement at the Regional Office for the Western Pacific



Annex 2. Procurement by supplier country or territory

Supplier Country/Territory	Total PO Value (in US\$)
Afghanistan	18 402 537.86
Albania	328 178.52
Algeria	1 240 515.50
Angola	1 446 664.46
Argentina	222 656.60
Armenia	902 163.30
Australia	5 213 854.04
Austria	14 492 344.62
Azerbaijan	315 484.97
Bahrain	34 232.90
Bangladesh	4 159 025.25
Belarus	87 666.28
Belgium	4 177 686.55
Belize	38 500.00
Benin	1 916 539.08
Bhutan	256 324.38
Bosnia and Herzegovina	417 213.90
Botswana	745 722.52
Brazil	1 169 255.91
Bulgaria	1 218 333.98
Burkina Faso	6 726 063.98
Burundi	1 729 458.14
Cabo Verde	337 678.49
Cambodia	1 241 196.39
Cameroon	4 835 596.46
Canada	6 829 371.26
Central African Republic	2 912 287.29
Chad	6 890 497.63
Chile	193 239.75
China	9 729 970.28
Colombia	154 744.97
Comoros	586 404.30
Congo	12 081 946.60
Cook Islands	6 322.43
Costa Rica	68 000.00
Côte d'Ivoire	3 538 662.55
Croatia	227 956.93
Cyprus	1 085 916.35
Czechia	1 157 950.06
Democratic People's Republic of Korea	64 065.50
Democratic Republic of the Congo	13 150 100.13
Denmark	66 654 665.12
Djibouti	1 092 323.81
Dominican Republic	11 200.00
Ecuador	198 719.00
Egypt	14 579 764.89

Supplier Country/Territory	Total PO Value (in US\$)
El Salvador	49 884.50
Equatorial Guinea	1 211 792.68
Eritrea	666 006.59
Estonia	502 323.85
Eswatini	1 127 542.79
Ethiopia	4 914 649.26
Fiji	3 895 259.99
Finland	427 422.46
France	20 047 056.62
French Polynesia	22 500.00
Gabon	906 357.39
Gambia	336 028.68
Georgia	1 213 801.56
Germany	15 586 101.46
Ghana	3 234 593.94
Gibraltar	3 800 252.60
Greece	5 636 062.03
Guam	32 296.78
Guatemala	101 590.00
Guinea	4 767 400.70
Guinea-Bissau	679 567.89
Haiti	47 812.00
Hungary	1 144 495.12
Iceland	10 205.09
India	64 836 686.24
Indonesia	2 223 632.26
Iran (Islamic Republic of)	417 257.39
Iraq	1 327 887.84
Ireland	17 935 054.12
Israel	2 443 343.82
Italy	10 474 837.44
Jamaica	16 000.00
Japan	2 801 388.12
Jordan	14 243 106.43
Kazakhstan	1 042 418.65
Kenya	12 789 607.00
Kiribati	1 147 092.58
Kuwait	71 441.01
Kyrgyzstan	2 024 535.57
Lao People's Democratic Republic	1 493 167.16
Latvia	191 618.94
Lebanon	10 682 244.70
Lesotho	733 545.79
Liberia	1 583 060.80
Libya	3 095 458.20
Lithuania	270 737.13
Luxembourg	723 069.23
Madagascar	2 320 982.16

Supplier Country/Territory	Total PO Value (in US\$)
Malawi	4 909 558.31
Malaysia	3 047 931.87
Maldives	824 341.00
Mali	2 385 979.03
Malta	333 691.18
Marshall Islands	43 670.37
Mauritania	1 338 270.06
Mauritius	888 007.36
Mexico	352 590.81
Micronesia (Federated States of)	150 664.28
Mongolia	2 261 576.26
Montenegro	131 867.92
Morocco	1 085 220.53
Mozambique	8 963 819.07
Myanmar	7 082 854.03
Namibia	631 490.05
Nauru	2 233.25
Nepal	3 510 130.31
Netherlands (Kingdom of the)	36 241 774.53
New Caledonia	54 939.54
New Zealand	1 208 338.76
Nicaragua	2 400.00
Niger	2 024 681.73
Nigeria	18 925 942.37
North Macedonia	327 103.11
Norway	3 147 757.18
Oman	745 227.34
Pakistan	51 437 108.50
Palau	23 010.00
Panama	40 000.00
Papua New Guinea	1 964 600.66
Paraguay	54 881.30
Peru	188 247.60
Philippines	8 103 153.88
Poland	587 797.13
Portugal	2 079 371.10
Qatar	96 037.32
Republic of Korea	2 980 477.02
Republic of Moldova	4 021 098.98
Romania	4 475 709.46
Russian Federation	1 256 786.09
Rwanda	2 877 140.02
Saint Lucia	44 000.00
Samoa	229 101.01
San Marino	33 000.00
Sao Tome and Principe	644 697.38
Saudi Arabia	385 185.05
Senegal	3 307 632.38

Supplier Country/Territory	Total PO Value (in US\$)
Serbia	1 035 861.06
Seychelles	210 258.64
Sierra Leone	3 993 688.33
Singapore	13 706 268.40
Slovakia	167 193.90
Slovenia	132 980.85
Solomon Islands	2 026 678.70
Somalia	8 579 012.75
South Africa	16 908 077.30
South Sudan	14 514 541.62
Spain	4 313 559.13
Sri Lanka	1 556 946.28
occupied Palestinian territory, including east Jerusalem	37 019 032.26
Sudan	14 699 758.57
Sweden	1 617 676.52
Switzerland	100 572 564.97
Syrian Arab Republic	3 654 742.09
Tajikistan	1 135 438.04
Thailand	1 303 385.72
Timor-Leste	1 742 455.36
Togo	1 893 405.75
Tonga	78 660.70
Trinidad and Tobago	64 500.00
Tunisia	3 017 180.20
Türkiye	11 242 297.41
Turkmenistan	512 875.76
Tuvalu	13 408.32
Uganda	5 902 832.62
Ukraine	34 993 755.05
United Arab Emirates	25 260 598.50
United Kingdom of Great Britain and Northern Ireland	30 853 072.80
United Republic of Tanzania	1 659 089.95
United States of America	42 840 689.57
Uruguay	48 778.00
Uzbekistan	1 088 187.83
Vanuatu	658 284.90
Viet Nam	1 754 995.87
Yemen	29 231 814.64
Zambia	2 354 067.40
Zimbabwe	2 384 024.91

Annex 3. How to become a supplier to WHO

Suppliers play an essential role in WHO's supply chain and in 2025 alone, WHO contracted over 12 935 different suppliers from 182 countries. General guidance on how to become a WHO supplier can be accessed on the WHO website (1), including the different procedures and solicitation methods used, contractual information, the WHO pre-qualification programme, and FAQ.

WHO participates in the United Nations Global Marketplace (UNGM) (2), the common procurement portal of the United Nations. UNGM acts as a single window through which potential suppliers may register to access a global market of over US\$ 29 billion annually across United Nations organizations. It allows suppliers to keep current business information available to all United Nations organizations as a tool for locating potential suppliers. It also provides online access to all tender opportunities and contract awards published by many United Nations agencies.

WHO issues tenders to its pre-qualified suppliers for goods and services directly and publishes open competition procurement opportunities on UNGM. While qualification criteria may be further specified to meet the needs of a particular purchase, all qualified WHO suppliers must meet the following general criteria:

- have adequate financial resources to perform the contract or the ability to obtain them;
- provide audited financial statements for completed fiscal years upon request;
- comply fully and effectively with WHO general terms and conditions of the contract;
- have the necessary organization, facilities, experience, accounting and operational controls; adequate insurance and technical skills (including, as needed, quality controls, property and production control systems, standards and safety programmes applicable to goods produced or services performed);
- have a record of satisfactory performance with WHO or UN Agencies, when applicable;
- have sufficient material and financial resources to meet all existing commercial commitments; and
- have not been temporarily suspended or debarred by WHO, another United Nations agency or the World Bank. WHO considers suppliers included in the United Nations Security Council Sanction Lists (1267) to be ineligible for WHO contracts.

New suppliers can register on UNGM and indicate their interest in doing business with WHO, and they can then compete for WHO procurement opportunities that are posted on UNGM. A step-by-step supplier guide is available on the UNGM and WHO websites including on how to use WHO's e-tendering system.

New suppliers may inform themselves of procurement activities of WHO and other UN entities using the Annual Statistical Report on UN Procurement data portal (3) on the United Nations Global Marketplace. This data portal provides access to procurement statistics of 39 UN organizations through interactive dashboards, as well as the annual reports themselves. This can be useful to suppliers to understand which goods and services are typically procured by each UN organization so that suppliers can target their commercial efforts. Potential suppliers may also check the lists of contracts awarded by WHO, which is included on the WHO website (4).

All WHO suppliers must abide by the United Nations Supplier Code of Conduct and comply with WHO General Terms and Conditions and other relevant WHO Policies available on the WHO website.

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